



D6.2 – Guideline for data users on good application and access practice

TEHDAS2 – Second Joint Action Towards the European Health Data Space

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0 Document info

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1 Executive summary

The European Health Data Space (EHDS) Regulation aims to harmonise and facilitate the cross-border secondary use of electronic health data. A key component is the common data access and request form, ensuring a common application procedure. Guidelines are developed to inform decision-making and aligned practices across member states, ensuring consistent, secure handling of electronic health data in compliance with EU regulations.

This guideline, based on the EHDS2 pilot project and its preliminary implementation in Release 3 of the EU central platform, provides practical, step-by-step instructions for applicants on the data access step of the [user journey](#). It covers:

- selecting the appropriate [application pathway](#) (data access vs. data request);
- [check list](#) for preparing necessary documents and information;
- introduction to the [application process and the application form](#);
- meeting [obligations](#) under the EHDS regulation;
- understanding [timelines, expectations and the role of HDABs](#) during the application process.

By following this guideline, applicants will:

- increase their chances of successful application by providing thorough, accurate information;
- facilitate faster and more efficient reviews by HDABs, reducing the time to obtain the data permit;
- ensure compliance with applicable regulations, including the EHDS regulation and the General Data Protection Regulation (GDPR).

2 List of abbreviations

Abbreviation	Description
Art	Article
CPU	Central Processing Unit
DPO	Data Protection Officer
EDI	Electronic Data Interchange
EDPB	European Data Protection Board
EEA	European Economic Area
EHD	Electronic Health Data
EHDS	European Health Data Space
EHDS regulation	Regulation (EU) 2025/327 of the European Parliament and of the Council of 11 February 2025 on the European Health Data Space and amending Directive 2011/24/EU and Regulation (EU) 2024/2847
EU	European Union
GDPR	Regulation (EU) 2016/679 of the European Parliament and of the Council of 27 April 2016 on the protection of natural persons with regard to the processing of personal data and on the free movement of such data, and repealing Directive 95/46/EC (General Data Protection Regulation)
GPU	Graphics Processing Unit
HDAB	Health Data Access Body
IBAN	International Bank Account Number
IDE	Integrated Development Environment
OECD	The Organisation for Economic Cooperation and Development
Peppol	Pan-European Public Procurement OnLine
RAM	Random Access Memory
SPE	Secure Processing Environment
VAT	Value Added Tax
VPN	Virtual Private Network
WHO	World Health Organisation

3 Introduction

3.1 Advancing health data use in the European Health Union

As part of the European Health Union, the European Union (EU) is advancing the use of health data for secondary purposes, including research, innovation and policymaking. Smooth and secure access to data will drive the development of new treatments and medicines and optimise resource utilisation—all with the overarching goal of improving the health of citizens across Europe.

TEHDAS2, the second joint action Towards the European Health Data Space, represents a significant step forward in this vision. The project will develop guidelines and technical specifications to facilitate smooth cross-border use of health data, and support data holders, data users and the new HDABs in fulfilling their responsibilities and obligations outlined in the EHDS Regulation.

TEHDAS2 focuses on several critical aspects of health data use:

- Collaboration models: developing guidance on collaboration and guidelines on fees and penalties as well as third country and international access to data.
- Data discovery: findability and availability of health data, ensuring it is accessible for secondary purposes.
- Data access: developing harmonised access procedures and establishing standardised approaches for granting data access across member states.
- Secure processing environment: defining technical specifications for environments where sensitive health data can be processed safely.
- Citizen-centric obligations: providing guidance on fulfilling obligations to citizens, such as communicating significant research findings that impact their health, informing them about research outcomes and ensuring transparency in how their data is used.

TEHDAS2 will contribute to harmonised implementation of the EHDS regulation through the concrete guidelines and technical specifications. Some of these documents and resources will also provide input to implementing acts of the regulation. Hence, the joint action will increase the preparedness for the EHDS implementation and lead to better coordination of member states' joint efforts towards the secondary use of health data, while also reducing fragmentation in policies and practices related to secondary use.

3.2 Target audience

The target audience of this guideline are health data applicants ("Applicant" or "Applicants") and health data users ("User" or "Users").

3.3 Purpose

The purpose of this guideline document is as follows:

- Provide essential information and guidance for completing:

- a well-founded “**data access application**” to access personal electronic health data for secondary use in anonymised or pseudonymised format;
 - a well-founded “**data request**” for an answer to health data in an anonymised, aggregated (non-personal) statistical format.
- Inform the applicants on the process to gain access to electronic health data for secondary use purposes, with that, enable applicants to make informed decisions before submitting a data access application or a data request.
 - Educate and raise awareness about applicants’ and health data users’ (from EU and EEA countries) rights and obligations throughout their journey from after the data discovery to receiving the permit for the health data use or health data statistical anonymised results, as well as using such data.
 - Provide the applicant with instructions on how to submit grievances or disputes in case their application is rejected or delayed.
 - **... and thereby support applicants in becoming health data users.**

4 Scope

This guideline is part of a series developed under TEHDAS2 to operationalise the EHDS regulation, specifically addressing Chapter IV on secondary use of health data. The scope of this guideline begins after the data discovery phase, once the applicant has identified the datasets of interest in the EU dataset catalogue and is ready to apply for the data via the EU central platform or any HDAB. See Annex 1: User journey for better insight into the described phases. The guideline covers the entire process of applying for health data access, including preparing necessary information, submitting the application and negotiating with the HDAB. It concludes with insights into the timeframes for different steps after access is granted and conditions of use after the permit.

It focuses on the data applicant role, providing:

- clear instructions for completing data access applications (for anonymised or pseudonymised individual-level data) and data request (for aggregated anonymised results);
- detailed guidance on preparing required information;
- an overview of applicant obligations and timelines.

All applicants and data requesters are strongly encouraged to familiarise themselves with the entire application process by thoroughly reviewing this guideline before initiating their access application or request. This comprehensive document aims to clarify and simplify the process of applying for or requesting health data through HealthData@EU.

5 Starting the application process for health data

This chapter provides key information for health data applicants before initiating a health data access application or request via the Data Access Application Management System (DAAMS) of the HDAB [1] or the HealthData@EU central service. Once the Applicant has identified the datasets of interest and collected them in the “dataset basket”, they are ready to begin the application process. At this stage, the Applicant must select the appropriate application path based on their needs. Under the EHDS regulation, the use of health data for secondary purposes is explicitly defined and the process distinguishes between two application types.

Health data access application (Article 67): for access to individual-level personal electronic health data. Data will be made available after anonymisation or pseudonymisation and will be accessible within a secure processing environment, to minimise re-identification risks. For the purposes of granting access to personal electronic health data, the HDABs shall assess whether all criteria are fulfilled (**Article 68**).

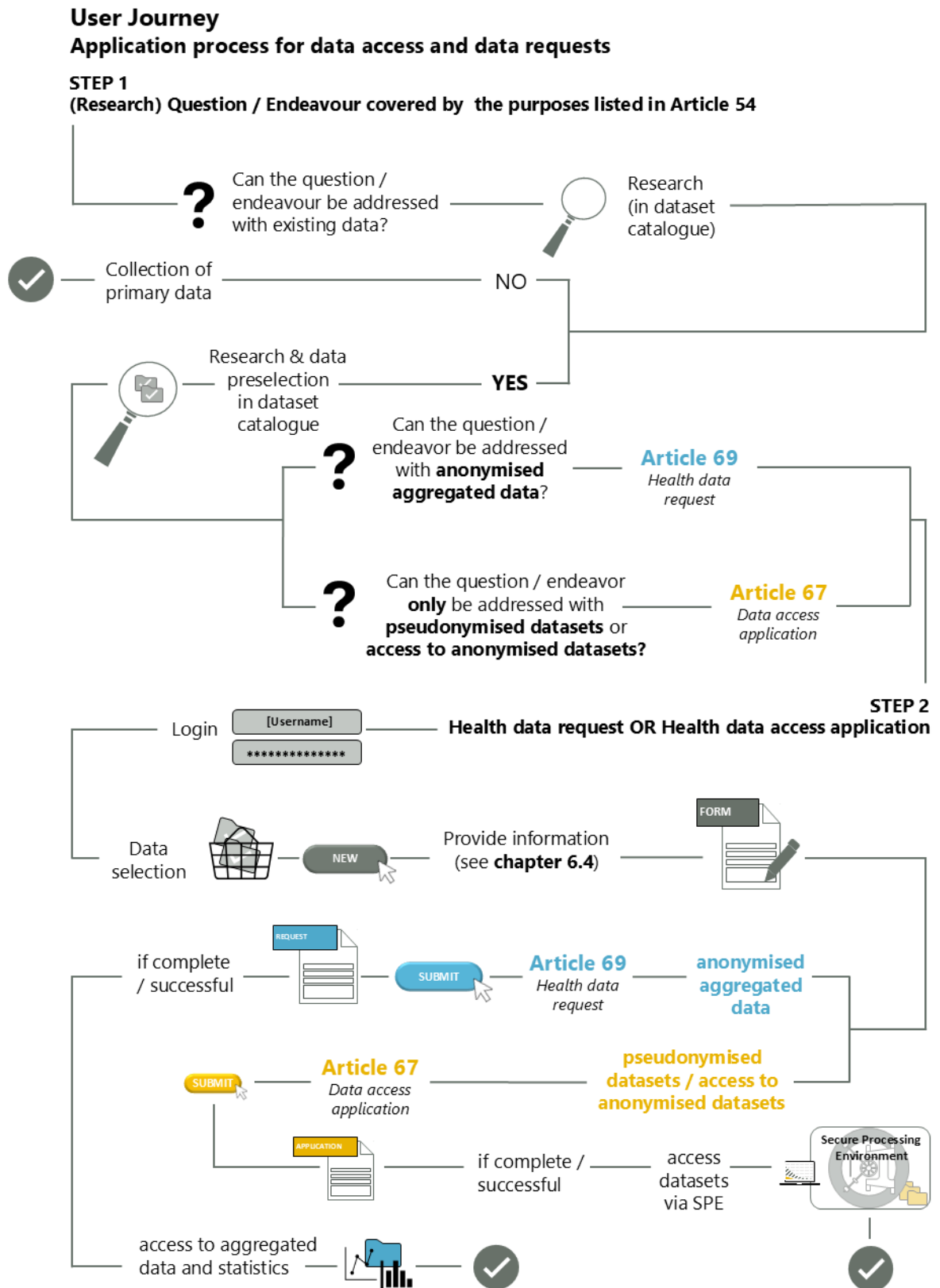
- **Example** of question requiring anonymised or pseudonymised personal electronic health data to be answered: What are the long-term health outcomes of patients with Type 2 diabetes who have undergone bariatric surgery compared to those who have not, considering individual patient characteristics such as age, gender and comorbidities?
- **Reason:** It requires detailed, patient-level data to analyse individual health outcomes and identify patterns or correlations based on specific patient characteristics to be answered.

Health data request (Article 69): for access to anonymised (aggregated) statistical results.

- **Example** of question requiring anonymised (aggregated) statistical results to be answered: What is the average incidence rate of Type 2 diabetes in the population over the past decade and how has it changed annually?
- **Reason:** It focuses on overall trends and incidence rates in the population, which can be addressed using aggregated statistical data without the need for direct access to personal electronic health data.

The applicant should be aware that requesting data under the EHDS involves a payable service, which includes the processing and handling of the data request, not just the final data result. Before proceeding, it is essential to assess your need and understand the documentation and justifications required for each path (see 5.6 5.6 for list of useful documents) and ensure compliance with applicable laws and principles (see Chapter 7).

Figure 1: Data user journey decision tree. Selecting the appropriate data application flow



This guideline relates primarily to the health data access application procedure. The simplified data request procedure is mentioned where it appears necessary.

5.1 Note on alternative access paths to EHDS infrastructure

Regulation does not prevent member states or data holders from offering other legal or administrative routes for accessing electronic health data. However, this guideline focuses solely on the EHDS procedure. Applicants choosing other access paths are advised to consult the relevant national authorities or data holders directly. In this case, the EHDS regulation provisions to facilitate data sharing do not apply. Thus, the data holder is fully responsible for acting on an appropriate legal basis for data sharing and informing you of the conditions. If you choose this way, this guideline is not relevant for you.

Benefits of using the EHDS and EHDS infrastructure include establishing transparent and common mechanisms for access to electronic health data for secondary purposes across Europe. Under the EHDS regulation, the EHDS supports GDPR [Articles 6\(1\)](#) points a., c., e., and f., in conjunction with **Article 9(2)**. Additionally, the EHDS infrastructure includes an overview of data sources, data holders, datasets, and data quality, a single path for applying/requesting necessary data from multiple data holders across nations, **predictability** as to where data will be made accessible for processing and terms of using the processing environment, and predictability of processing time, conditions, and fees to be expected.

5.2 Which application path?

HDABs prioritise the protection of natural persons by granting access to anonymised statistical data wherever possible (health data request). For data access applications, anonymised data is the default option and HDABs will only make pseudonymised data available if it is essential for the intended purpose (**Article 66(2)-(3)**). Applicants will need to provide a clear argument if pseudonymised data is required (**Article 67(2) point e.**). Selecting an incorrect path to achieve the given objectives may result in delays or additional scrutiny. Based on the information provided in a data access application or data request the HDAB will either grant or reject the application or request and determine the specific restrictions on the data usage, e.g. dissemination level.

If a data access application or data request is rejected, a detailed explanation of the reasons will be provided (**Article 68(9)**). The decision can be appealed and a structured process for resolving disputes between stakeholders is available. Transparency and fairness throughout this process are ensured by the EHDS regulation.

5.2.1 Health data request

Applicants should choose this path if anonymised aggregated statistical results meet their objectives. This path is ideal for analyses where there is no need for direct access to personal electronic health data (e.g. to assess population-level trends or summary statistics) or for querying datasets to obtain further anonymised aggregated details on its contents (e.g. how many individuals with mutation X are present in dataset D) and there is no need for the data user to access the datasets themselves (**Article 69**).

Key features include:

- applicants receive anonymised, aggregated outputs generated by the HDAB or trusted health data holder;
- no pseudonymised or anonymised personal data is shared with the applicant;
- the HDAB (or trusted data holder) remains responsible for processing of the individual-level data and provides only anonymised statistical results to the applicant, without disclosing personal or pseudonymised data;
- this pathway is suitable when aggregated statistical data is sufficient to meet the applicant's objectives.

Examples:

- If a policymaker needs high-level aggregated statistics, such as the number of cancer diagnoses in a region, they will submit a data request. This approach provides summary statistics and avoids direct access to personal electronic health data.
- For a researcher aiming to determine the prevalence of diabetes and hypertension in a specific age group across member states, a data request can provide anonymised counts of individuals meeting these criteria. This would help the researcher assess whether a more detailed data access application is necessary.

Tip: In some cases, applicants may be able to contact the HDAB (or relevant national contact point) or data holders before submitting their application to obtain additional information about the dataset of interest. This possibility depends on national implementation and may support applicants in assessing the feasibility of their request (e.g. sample size, data availability), prior to formal submission.

5.2.2 Data access application

Applicants should select this path only when individual-level personal electronic health data is necessary to achieve the objectives and aggregated statistical results are insufficient to meet the needs. Users can work with the data in an SPE when granted access. This path can apply to two types of datasets under the conditions stipulated in the EHDS regulation.

1. Pseudonymised datasets: when anonymised data is insufficient to achieve the objective of the processing and a clear justification is provided.
2. Anonymised datasets: when direct access to anonymised datasets is sufficient for the intended analysis and/or pseudonymised data is not considered by HDAB to be needed for the described purpose.

Applicants must explain why the data applied for (in anonymised or pseudonymised format) is necessary for their purpose and demonstrate compliance with data minimisation principles. The HDAB evaluates applications for both compliance and necessity. All electronic health data can be accessed only within an SPE, ensuring strict data protection and preventing unauthorised re-identification. Further information on SPEs within the framework of the EHDS regulation and their use can be found in the respective TEHDAS2 Guideline 7.1 [2].

Example: If a researcher needs to study the long-term impact of diabetes treatments across multiple member states, they may require pseudonymised data to link patient records from national registries. Aggregated data would not suffice, as the researcher needs to track individual treatment outcomes over time. The application must justify why pseudonymised

data is essential and confirm adherence to GDPR and the EHDS regulation requirements, including data minimisation.

5.3 Who can apply?

Under the EHDS regulation, any natural or legal person may apply for access to electronic health data for secondary use for lawful purposes (**Article 67(1)**). This only applies for EEA's persons and those established in a third country that is recognised as providing reciprocal access to EU-based data applicants to data held by holders in that third countries via an implementing act pursuant to **Article 91(2)**, or where they are established in a third country that has become an authorised participant in HealthData@EU pursuant to **Article 75(5)**. However, please note that this possibility for designation will only apply after a long transitional period of 10 years from the entry into force. The right to apply does not guarantee that access will be granted.

Potential data applicants are:

- public authorities
- healthcare providers
- researchers and research institutions
- private entities and companies
- developers of AI systems for healthcare
- educational institutions
- national health care authorities
- natural persons.

If the application process is successful, the applicant becomes a health data user and therefore must fulfil certain obligations. These are outlined in **Article 61** of the EHDS Regulation and further detailed in 7.1 .7.1

These obligations also include the publication of results, which are described in detail in [3].

5.4 Allowed purposes for secondary use of electronic health data

Access to data categories specified in **Article 51** for secondary use should benefit society, such as through research and development of new medicines, medical devices and healthcare products and services at fair prices for Union citizens (**Article 53**). It should also enhance access to these products and services across all member states.

Lawful purposes for processing electronic health data for secondary use under the EHDS regulation (**Article 53**) include the following:

- a. **Public interest – protect against health threats and ensure quality and safety of healthcare.**
 - Choose this purpose if:
 - you aim to monitor, prevent or respond to health threats, such as infectious disease outbreaks or environmental health hazards;

- you need to study trends in vaccination uptake, disease prevalence or health outcomes for specific populations.
- Example: Monitoring vaccination coverage to identify areas with low uptake and implementing targeted campaigns to boost vaccination rates.

b. Policymaking and regulatory activities: Support public bodies – help government and EU health institutions.

- Choose this purpose if:
 - your work supports policymaking, resource allocation or other administrative functions of public health institutions;
 - you plan to assist government bodies in designing evidence-based healthcare policies or initiatives.
- Example: Analysing hospital admission trends to help a health ministry allocate resources during a health crisis.

c. (Official) Statistics – create health-related statistics.

- Choose this purpose if:
 - you aim to generate anonymised health statistics for use in reports, dashboards or publications;
 - you are producing summary-level insights, such as disease prevalence or health service usage rates.
- Example: Producing annual statistics on chronic disease prevalence across member states, segmented by demographic factors.

Purposes referred to in points a. to c. are reserved for public sector bodies and Union institutions and third parties acting on behalf of a public sector body or Union institution (**Article 53 (2)**).

d. Education – for teaching in health and care.

- Choose this purpose if:
 - you plan to develop educational materials or training for healthcare professionals or students;
 - you want to create anonymised case studies for use in academic or professional healthcare programs.
- **Example:** Developing simulated case scenarios for training medical students in diagnosing rare diseases.

e. Scientific Research related to health

- Innovation – develop new health products and services.
 - Choose this purpose if:
 - you are conducting research to create new medical devices, therapies or healthcare technologies.
 - **Example:** Using pseudonymised data to study genetic markers for rare cancers and develop precision medicine approaches.
- Algorithm testing – train and test medical algorithms.

- Choose this purpose if:
 - you aim to train, validate or refine machine learning models or other algorithms in healthcare.
- **Example:** Training an algorithm to detect early signs of heart disease using anonymised health records.
- (Public) Health Research
 - Choose this purpose if:
 - you are conducting register-based research aiming to identify risk factors for diseases, social disparities in health and/or conduct epidemiological surveillance etc.
 - **Example:** Using pseudonymised data to identify populations at risk for a certain disease and develop strategies to protect them
- f. **Personalised healthcare – provide tailored healthcare based on others' data.**
 - Choose this purpose if:
 - you are studying ways to personalise treatments or healthcare interventions based on patient-specific factors, such as genetics or comorbidities.
 - **Example:** Analysing pseudonymised health records to identify optimal treatment plans for patients with specific genetic profiles.

5.5 Prohibited purposes for secondary use of electronic health data

Under the EHDS regulation any secondary use of electronic health data that falls outside approved purposes is prohibited, especially if it negatively affects individuals or groups. See below the purposes prohibited according to **Article 54**.

- a. Detrimental decisions – decisions that negatively impact individuals based on their health data.
- b. Insurance discrimination – decisions to exclude or alter insurance benefits or premiums based on health data.
- c. Advertising – advertising or marketing to health professionals, organisations or individuals using health data. Use in congresses is allowed if not promotional¹.
- d. Unauthorised access – sharing health data with third parties not specified in the data permit.
- e. Harmful products – development of products or services that could harm individuals or society, such as illicit drugs or items against public order or morality.

Detailed guidance on completing the “Purpose” section of the application form is provided later in this document, see 6.4.5 6.4.6 . Applicants are encouraged to refer to that section for step-by-step instructions and examples.

¹ The prohibition on advertising includes the use of EHDS-derived data for promotional or commercial purposes, such as product marketing campaigns or commercial brochures. However, the scientific dissemination of research results, including those presented at scientific or medical congresses or published in peer-reviewed journals, is not considered advertising. Use of EHDS data in medical affairs materials must be non-promotional, evidence-based, and strictly aimed at education or scientific exchange, in line with applicable EU legislation and ethical standards.

5.6 What documents and information to prepare before filling in the application

Standardised electronic forms are provided for processing health data access applications and data requests. These forms ask for various formal criteria, which applicants can usually prepare for. To ensure that the application process runs as quickly and smoothly as possible, it is advisable to follow the checklist below to have the information and documents ready before starting the application process. The respective fields of the application form are described in detail in the section [Data access application form](#).

- Project description for public announcement
 - A summary of the project that can be shared with the public by the HDAB on their website within 30 working days after issuance of the data permit or reply to a data request. Should not include any confidential information (**Article 58**).
- Summary of the project plan for decision making purposes
 - Maximum two pages long, written in one of the official EU languages.
 - This document will not be made public and thus can include also confidential information.
 - Payment details ([Section 4](#) of the application form) for the person and organisation that will receive the bills related to this application. Include whether they can receive e-invoices, as well as the project number and cost account.
- Detailed data description that the applicant needs for the project/study
 - Include specifics of the datasets needed for the project or study such as list of variables required, data format (e.g., CSV, Excel, JSON), time period (e.g., data from January 2020 to December 2021), or level of the data aggregation (e.g., aggregated data by region).
- Justification on why the data access is needed in case of data access application to individual-level data ([Section 5](#) of the application form)
- Expected public benefits of the results based on the data applied for ([Section 5](#) of the application form)
- Approvals depending on the source of the data
 - information on any assessment of ethical aspects of the processing, required under national law, which may serve to replace the health data applicant's own ethics assessment may have to be provided and can be attached to the application (**Article 67(2) point j**) ([Section 6](#) of the application form).
- In a case of combining existing data in applicant's possession with the dataset of interest, the applicant shall have prepared the following to be provided in the application form to the HDAB (For applicants with existing datasets, [Section 7](#) of the application form):
 - Prior consent or permit to existing datasets that the applicant already has from the HealthData@EU infrastructure or outside the infrastructure.
 - Consent is required as a legal basis if the data which are supposed to be combined with the datasets of interest were initially collected based on consent.

- If the data in the applicant's possession were collected under a prior permit (outside the HealthData@EU), they must provide documentation of that permit, including details such as the issuer, date, validity period and any identifying information (e.g., permit code).
- Prepare the SPE machine specifications depending on the complexity and volume of operations ([Section 8](#) of the application form). Applicants will be asked to specify:
 - The memory needed for their analysis (e.g., RAM).
 - The number and type of processors required (e.g., Processing Units [CPU/GPU]).
 - Adequate space for both data and software.
 - Operating system compatible with their software and tools.
- **Review Article 68** about data permit for complete list of criteria that must be met to obtain a data permit. Applicants should ensure that their application meets all criteria listed in **Articles 67 and 68**.

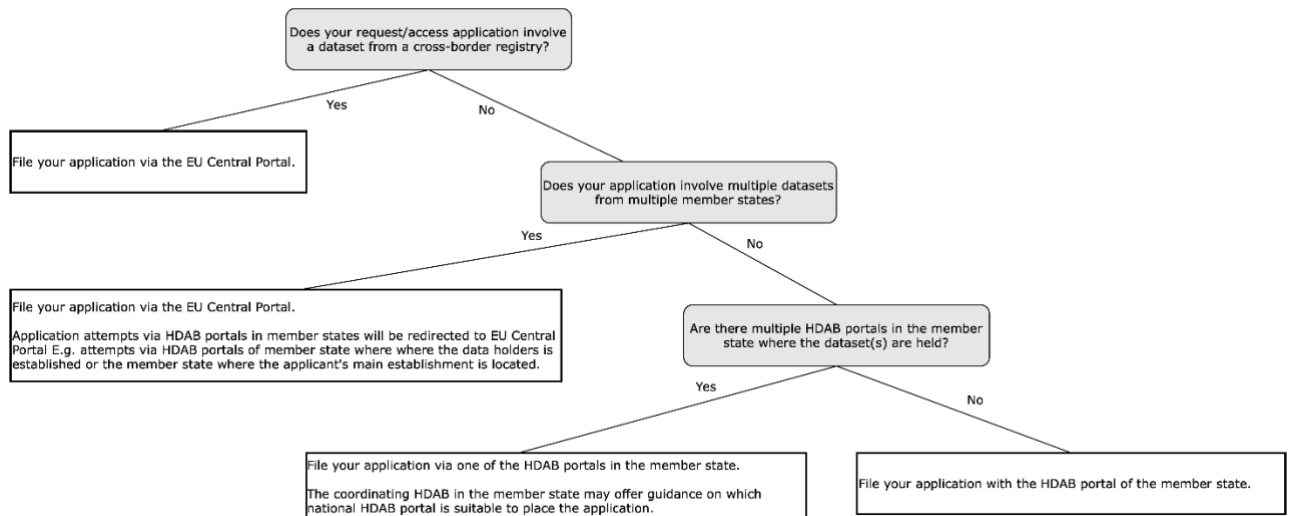
6 How to apply for dataset(s) of interest

In the data application phase, the health data applicant applies for access to the individual-level electronic health data or requests aggregated, statistical results counted by HDAB. To do so, data applicants should provide all the necessary information in an application or request for it to be reached to the relevant HDABs. All datasets of interest must be listed in the EU catalogue for application to proceed. If not listed, the HDAB must request compliance from the data holder².

The nature of datasets within an application determines the suitable HDAB portal to place the application. The choice of the HDAB portal will be based on whether the application includes data from cross-border registries (6.1), data from multiple countries (6.2) or data from a single country where they may or may not be multiple HDAB portals (6.3) applicants may use the decision tree provided in Figure 2 to choose the most appropriate portal to file their application.

Figure 3: Decision tree on which portal to use for health data application submission. National portal(s) or EU central portal to be selected based on the results. *Grey background*: question to answer; *white background*: possible answer to the question. Based on EHDS **Article 67(3)** and recital (80).

² Under the EHDS Regulation, datasets eligible for secondary use must be described in the EU dataset catalogue. Therefore, applications should refer to datasets listed in the catalogue. However, in rare cases where a data holder has not fulfilled this obligation, and a dataset of interest is not yet published, it is the responsibility of the relevant Health Data Access Body (HDAB) to request the dataset's inclusion in the catalogue before proceeding with the application process. This ensures legal and procedural compliance.



6.1 Cross-border registries

Cross-border registries represent a particular source of data. These registries receive data from different healthcare providers in several member states; an example is the European Reference Networks for Rare Diseases. To make an application for a dataset that is provided by such a cross-border registry, applicants must submit their applications via the central EU platform. The application will be forwarded to the HDAB in the member state where the coordinator of the registry is located, and this HDAB manages the application (**Article 76**).

6.2 Multiple datasets in multiple member states

The EU dataset catalogue, accessible through the central EU platform, is a federated catalogue composed of national catalogues maintained by health data access bodies (HDABs). Each HDAB publishes and maintains its dataset descriptions locally, and transmits this metadata to the central EU platform, which consolidates and displays them in a unified interface for users (**Article 79**).

The EHDS Regulation (**Article 67(3)**) allows applicants to submit a multi-country application either through a national HDAB or via the central EU platform. In practice, any attempt to search for or request datasets from more than one member state via a national HDAB portal will result in automatic redirection to the central EU platform. This ensures consistency, avoids duplication, and guarantees that multi-country applications are handled via a single, harmonised workflow.

As a result, applicants are encouraged to start their discovery and application process directly on the EU central platform when intending to request datasets from multiple countries. For more details see [4] or [5].

6.3 National datasets from one member state

For datasets located in a single member state, applications must be submitted to the national HDAB in that member state. In the case of application to multiple datasets from different national HDABs, the coordinating HDAB is the most relevant place to apply (**Article 55(1)**).

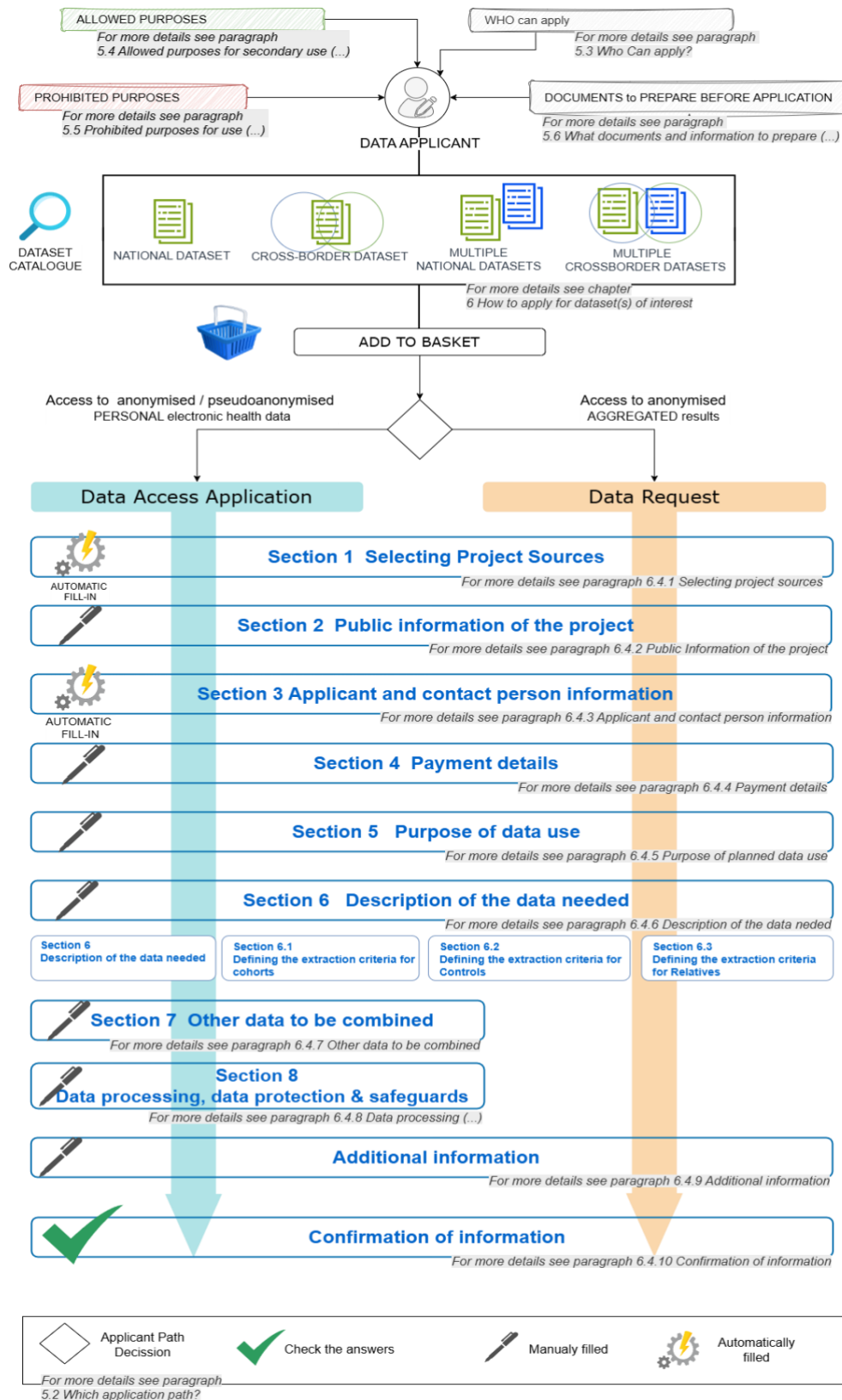
6.4 Data access application form

The data access application form is a critical part of the application process. It ensures that the HDAB can evaluate applicant's needs based on the principles of the EHDS regulation. This form also aligns with the data request form, although the latter is shorter and contains fewer questions (see Figure 4: Data Access Application Form flow diagram. Blue arrow for the health data access application, orange arrow for the health data request flow. Additional information for each step is referenced under each one (grey background). Based on *HealthData@EU central services DAAMS*. Figure 4). During the application process, HDAB portals will prompt applicants to complete the respective forms. Although the order of form sections or visual may vary between portals, the minimum information requirements are consistent and outlined [4]. This guideline section provides guidance on the access application form as implemented by the EU central platform.

The access application and request forms can be completed in any official EU language³. After selecting the datasets and adding them to the “dataset basket” it is possible to create a new data application or data request.

³ https://european-union.europa.eu/principles-countries-history/languages_en

Figure 4: Data Access Application Form flow diagram. Blue arrow for the health data access application, orange arrow for the health data request flow. Additional information for each step is referenced under each one (grey background). Based on *HealthData@EU central services DAAMS*.



Tip: Applicants shall name their application as descriptive as possible in the first step. A precise naming practice serves the applicant. Applicants shall consider including, in the

name, as a minimum their research question and target population. It should be noted that application title will not be evaluated by the HDAB or affect in the application process.

Once the application is created, applicants will be redirected to the application form itself. The form consists of 10 sections and each of them consists of a different number of questions/input fields, see Figure 4.

The form and the respective sub-sections consist of different types of questions and input requests which essentially differ in the ways the answers should be provided. Mandatory information is always marked with a red asterisk (*), regardless of the required answering format. Depending on the respective answers, the form can branch off in different directions, for example, if pseudonymised data processing is requested, more detailed information is required from the applicant. For some examples of key input types in the form see Annex 4: Type of Input Fields in the Common Access Application Form.

The sections of the application form are described in more detail in the following subchapters. Applicant will get familiar with the practicalities that should be fulfilled in each section before working with the form online.

6.4.1 Selecting project sources – Form Section 1

About this section

Data users should only apply for data that are adequate, relevant and limited to what is necessary in relation to their purpose of processing indicated in the health data access application (**Article 66**), following the principle of data minimisation of the GDPR ([Article 5\(1\) point c.](#)). The health data access body evaluates carefully if your requirements are in line with the GDPR data minimisation principle.

Why: This section is dedicated to identifying the data sources relevant to the applicant's needs.

What: Applicant shall specify the sources of data needed. Once they select the datasets, the field will be filled in automatically.

Examples: This could include registries, biobanks, hospital databases, clinical trial datasets, public health records etc. but only when offered via the dataset catalogue.

Tips: For multi-country projects, applicants shall ensure data from each country are covered. Details for each dataset will be then required in [Section 6](#) of the application form.

6.4.2 Public information of the project – Form Section 2

About this section

The HDABs in the European Union are obliged to publish information on the data permits, requests and applications (**Article 57(1) point j.**) on their website within 30 working days after issuance of the data permit or reply to a data request. In this section, you are asked to provide information on your project that can be shared with the public. Make sure this does not include any confidential information. Provide your answers in layperson's terms.

Why: To comply with public disclosure requirements. This section is dedicated to information on the project that can be shared with the public.

At the application stage, the form collects information about the applicant's plans with the dataset(s), as they are not yet a data user. Once the data request or access application is approved and the applicant becomes a data user, they have additional reporting obligations on project outcomes. These are detailed in Section 7.5 of this guideline.

What: Provide a general overview of the project that is fit for public disclosure, regarding aspects such as:

- Project name* (max 500 characters);
- Project leader name* (max 500 characters);
- Country of the project leader*;
- Purpose for which the data will be used*;
- The research focuses on the following objectives*;
- Area of research*;
- Description of the data they will use (open question) * (max 2000 characters);
- Summary of the project* (max 2000 characters).

Examples:

Public interest – patient safety: “This project aims to improve patient safety by analysing anonymised data on adverse reactions to newly introduced vaccines, collected from vaccination registries and hospital databases in Sweden, Denmark, and Finland. The study will focus on identifying patterns and risk factors associated with adverse events reported between 2015 and 2025. By understanding these trends, healthcare providers can enhance vaccine safety monitoring systems and develop guidelines to mitigate potential risks, ensuring safer immunisation practices.”

Public interest – threats preparedness: “This project focuses on using anonymised health data from past epidemics, including H1N1, seasonal influenza, and COVID-19, collected from Spain, Portugal, and Greece. By examining data on infection rates, healthcare utilisation, and patient outcomes from 2009 to 2021, researchers aim to identify key factors that influenced the spread and severity of these diseases. The findings will help develop more effective strategies for early detection, containment, and treatment of future health threats, enhancing overall preparedness and response capabilities.”

Policy making: “This project aims to assess the long-term effects of smoking bans implemented in public places across Germany, France, and Italy. Using anonymised health data from national health surveys and hospital records from 2005 to 2020, the research will analyse trends in respiratory diseases, cardiovascular conditions, and overall mortality rates. The study seeks to determine the effectiveness of smoking bans in reducing health risks

associated with tobacco use and to provide evidence-based recommendations for future public health policies.”

Scientific research related to health care: "This project aims to study the long-term effects of chemotherapy on cancer survivors, using anonymised data from the national cancer registers and hospital databases across the Czech Republic, Austria and Netherlands. The research will focus on understanding how chemotherapy impacts the quality of life, mental health and recurrence rates in survivors of breast and colorectal cancers. By analysing patterns in treatment outcomes over a 10-year period covering the years of 2010-2020, the project seeks to identify factors contributing to improved recovery and long-term health. This study will also explore biological sex-specific differences in post-treatment outcomes and inform future clinical guidelines for personalised care of cancer survivors."

Tips: Pay attention to not include any confidential information. Provide answers in layperson’s terms.

6.4.3 Applicant and contact person information – Form Section 3

About this section

A data permit application should state the name and contact details of the applicant, either legal or natural person. Information on the contact person responding to any inquiries related to the application, be it the same person as the applicant or another person, should also be included. If the contact person is not the same person as the applicant, their relationship, e.g. based on an employment contract, should be clarified.

Why: To facilitate communication with the HDAB. Information on the contact person responding to inquiries related to the application will enable better communication with the HDAB.

What: Provide detailed information about the applicant and the primary contact person. The information can be filled in automatically from the applicant’s user profile, if available.

Example:

- **Applicant (Legal person):**
 - Full name*: University of Health Research
 - Street name and number*: 25, Daisy Street
 - Zip Code*: ZP 012345
 - City/Town*: Acme Town
 - Country*: Country X
- **Contact person:**
 - Full name*: John Smith,
 - Email*: john.smith@healthuniversity.eu,
 - Phone*: +123456789.
 - Name of the organisation*: University of Health Research,
 - Business ID of the organisation*: BID12345
 - Job title*: Data Coordinator

- Affiliation*: Department of Population Health Studies
- What is the relationship between the contact person and the applicant? *: Employee of the university.

Tips:

- An applicant could be either a legal or a natural person.
- What if the applicant is not a single legal or natural person? If you are, for example, a consortium of universities and research institutions from different EU member states, please decide who is the lead applicant and fill in the application accordingly.
- If the applicant is a legal person then the contact person is a natural person that has some affiliation to the applicant.
- If the applicant is a natural person, they are also considered the contact person for the application.

6.4.4 Payment details – Form Section 4

About this section

Payment details of the person to whom the HDAB addresses the bills related to this application and the consequent data permit, if granted.

Why: Enabling billing for fees related to the application. The fees are used to cover the work to assess applications, prepare datasets and provide access to SPEs (**Article 62**). More details about fees can be found in 7.6 .

What: Provide information on payment details of the person and/or organisation to whom the HDAB addresses the bills related to this application and the consequent data permit, if granted.

Example:

- Full Name *: Jane Doe
- Postal Address *: 123 Health Drive, Suite 45x, Wellness City, Country
- Phone Number *: +123 456 789
- Email *: jane.doe@example.com
- Invoice Type (Paper/Electronic) *: Electronic
- Invoice Reference Number *: 789XYZ123
- E-invoice Address (IBAN) *: FI21 1234 5600 0007 85
- Operator ID *: OP123456
- Name of the Organisation *: HealthData Inc.
- Business ID of the Organisation *: 9876543210
- VAT Number *: FI9876543210
- Peppol Code (if applicable): 0192
- Is the Project Financially Covered? *: Yes
- Range of Amount of Financing *: 50,000 - 75,000 Euro

Tips:

- Choose the preferred invoice type carefully. If electronic, ensure EDI or IBAN and operator ID details are correct.
- Clearly state if the project is financially covered. Be transparent if funding is not yet confirmed.
- If unsure about details like Business ID, VAT number, or Peppol code (Identifier for Pan-European Public Procurement Online), applicants shall consult with their financial team and promptly update any changes in contact and/or financial information before the submission to avoid miscommunication and delays.
- Ensure that the information provided matches supporting documentation they have provided with the application, such as contracts or financial agreements. Inconsistent details can cause complications.
- Carefully review the entire section before submitting to catch any potential errors or omissions. It can save time and prevent delays in the application process.
- The Invoice Reference Number is not necessarily a number but any information how to make sure the invoice is correctly routed at the invoice recipient.

6.4.5 Purpose of planned data use – Form Section 5

About this section

Applicants should indicate the purpose for which data are sought (**Article 53**). Applicants need to explain and argue why the requested data are necessary for their indicated purpose of use. Applicants are also asked to provide information on the aim of the project. Then, depending on the use purpose applicants need to provide a summary of the plan for using the data or a summary of the research plan, and information on the person responsible for the data use or research

Why: This section aims to collect the information to evaluate whether the requested data can be ethically and technically provided for the planned purpose, ensuring alignment with EHDS-approved purposes and data minimisation principles, as well as the ethical implications of the planned use.

What: The application or request should state clearly which purpose the health data is to be used for. This is essential as any secondary use must comply with EHDS approved purposes (see 5.4 5.4). Any planned purpose not complying, will be denied/rejected (see 5.5).

Applicant must justify the necessity of the data and its amount (max 2000 characters), describe their project's aim (max 1000 characters), expected benefits including societal impact and provide details about the person responsible for data use (max 2000 characters).

Additionally, applicants need to specify the format of the electronic health data (anonymised or pseudonymised) and they will need to submit a summary of their research plan as a document attachment via Additional Information [Section 9](#) of the form. National HDABs may also offer further guidance on the expected contents of research plans that are part of the applications.

Examples:

Policy development: “This project supports the development of a national strategy to improve access to mental health services. By analysing anonymised data on service utilisation and patient outcomes from 2015–2024, the Ministry of Health aims to identify geographical inequalities and inform targeted investment in underserved regions.

Healthcare quality improvement: This project evaluates the impact of a new digital triage system in emergency departments. Using aggregated data on waiting times, diagnosis accuracy and referral patterns, the project will support continuous improvement of care delivery pathways in hospitals.

Below is an example justification of the need of personal electronic health data in pseudonymised form:

Personalised medicine: “This project aims to explore sex differences in the management and outcomes of Type 2 diabetes in individuals aged 18–65 years across three member states. By analysing longitudinal, personal, pseudonymised health data, we will assess how treatment adherence and comorbidities affect long-term health outcomes, such as cardiovascular events. Anonymised data are insufficient, as we require linking multiple datasets across national registries at individual-level, which necessitates pseudonymised data to maintain continuity between observations. All data will be processed in an SPE with strict access controls, ensuring compliance with GDPR and EHDS principles. Results will inform clinical guidelines for personalised diabetes care and support policymaking on chronic disease management.

Tips on how to determine the use purpose

- Refer to the list of permitted purposes provided in the application form.
- Clearly state why anonymised or aggregated data is insufficient for your objectives, if applicable.
- Ensure the intended use aligns with the regulation by explaining how it contributes to public health, research or societal benefits.
- Do not use health data in ways that could have negative social, ethical or economic implications.

6.4.6 Description of the data needed – Form Section 6

About this section

In this section, you need to provide a description of the requested dataset record, clearly indicating which dataset records the application concerns. You should only apply for data that is adequate, relevant and limited to what is necessary in relation to your purpose of use, following the principle of data minimisation of the EU’s General Data Protection Regulation ([Article 5\(1\) point c](#)). The health data access body evaluates carefully if your requirements are in line with the GDPR data minimisation principle.

Why: Trusted health data holders and the HDABs (from different member states) evaluating the application need to understand the data needed/applied for. The description provided in

this section is necessary for the HDABs to assess whether the application is in line or not with GDPR and the EHDS regulation.

What: Provide a description of the required data for each country/HDAB that is a provider of the datasets that applicant have included in their application.

Figure 5: Screenshot for section 6 – application involving multiple datasets from multiple countries/HDABs

European Union	Belgium	France	Finland
Section 6 0/2	Section 6.1 0/15	Section 6.2 0/1	Section 6.3 0/1

As an overview of the data being applied for, the application form will display, per country, the list of the data included in the application and the list of HDAB(s) that will receive the application. In form Section 6 the following information is collected:

- If the applicant has already been in contact with the respective HDABs, they should provide the contact details; this information would help the application be forwarded to the appropriate persons (max 2000 characters).
- Applicant should also describe their data linkage plan, meaning how they will link data they obtain from various sources, e.g., based on a personal identification code or through subject similarity metrics *(max 2000 characters). More information about data linkage can be found in [6].

Defining data extraction criteria – Form subsection 6.1

Applicant will be prompted to define the criteria for their **study cohort**. This is crucial as the data holder will use these criteria to prepare the necessary datasets.

The study cohort can be established using:

- criteria from the application form;
- a previously established cohort (e.g., from an earlier healthcare-research study);
- a combination of these options;
- the entire population of a country, state or other area.

For all these options, applicant will need to answer a core set of questions.

- If applicant has an ethical review from a national or institutional ethics board for the project, they should include it in this section if relevant for the use-case. Note that local approval does not apply for another country.
- Specify the data variables to be used during extraction by referring to the descriptions in the data holder's metadata catalogue or data dictionaries (max 2000 characters). In addition, **variable lists and value sets** can be provided as document including additional information ([Section 9](#) of the form).

- Indicate the size of the required study cohort (exact or estimated) and justify this size (max 2000 characters).
- Specify the period for the data needed (max 2000 characters).
- Describe the required extraction method (random sample, all qualifying individuals, or other) and provide the inclusion and exclusion criteria (max 2000 characters respectively).
- If the data will be extracted **multiple times**, specify the required frequency or time points (max 2000 characters).
- If the order in which data are extracted from different data holders affects the composition of the cohort, please specify the expected sequence of extraction (max 2000 characters).
- If applicant's **study cohort is based on a previously established cohort**, then the applicant will need to provide details on this prior cohort. Specifically, whether the cohort has been formed based on informed consent, detailed description of the legal documents that underpin the previous cohort formation, including permits and/or informed consent and finally they will be asked to identify the contact person who will be responsible for delivering the study cohort information to the relevant HDAB. This person could be, e.g., the principal investigator/clinician from the previous study or a contact of the Trusted Third Party with access to the prior study's cohort composition.

Groups of controls and relatives – Form subsections 6.2 and 6.3

Among the extraction criteria the applicant defines for the study cohort, they can:

- **re-use** the data variables, required time periods, extraction method, extraction periodicities and extraction order for the controls and relatives;
- they can **re-define** the criteria for these groups if needed.

Also, for controls and relatives, applicant will be asked to specify the size of these groups and their inclusion and exclusion criteria and information on previously issued permits (if any) that serve as the basis for the formation of control and relative groups.

Examples of data minimisation:

- Age ranges of individuals (e.g., 18-21 years, 21-24 years, etc.) instead of exact ages and dates of birth. If knowing the exact age or month of birth is essential, then applicants shall provide justification. Additionally, applicants shall specify which variable requires the exact age information.
- Among multiple attributes that could serve the same purpose or could substitute each other, request only one. For example, patient's blood pressure readings or their heart rate. Both attributes can provide insights into the patient's cardiovascular health, but to serve the purpose of assessing their general health status only one is needed.
- Time ranges instead of exact time points e.g., month of clinic visit instead of exact date.
- Racial or ethnic origin related information shall be included only when the lack of that information would make applicant's study goals unachievable.
- Data sample when, e.g., a whole population is not necessary for realising applicant's aims.

Tips:

- Apply only for data that are adequate, relevant and limited to what is necessary in relation to the purpose of use. Do not request data variables for which they would not be able to justify its necessity for the project. Applicants shall ask themselves “why do I need this variable?” - and if they do not have a valid response, they shall leave that variable out from their application. A response such as “it might be useful later” is not in line with data minimisation (**Article 66**).
- Ensure the amount of data they request is adequate but not excessive.
- Be aware that if a data user requests changes after the permit has been issued (e.g., extending the permit duration or modifying the dataset), additional fees may apply to cover the costs of these adjustments. Detailed information on how to use data in an SPE can be found in [2].
- Consider the privacy impact of each data element.
- Request only the necessary number of records. If an applicant needs data for an entire country's population, they shall clearly justify why data of this scope are required.
- Check with their national data protection authority on the possible ways of implementing GDPR requirements on data subjects' rights, concerning particularly of rights to information. If applicants are organisationally affiliated, they shall request support from their data protection officer (DPO) if their organisation has one.
- Additional information and instructions on data minimisation can be found in [7].

6.4.7 Other data to be combined – Form Section 7

About this section

It is possible to combine other data, such as data in your possession or data obtained from elsewhere, with the data applied for with this application. List the information for all additional data that will be combined with the data authorised by the HDAB. If you later want to combine data with the data authorised by the HDAB, you need to submit an amendment application to the same health data access body. If you have any other data in addition to the study cohort that you would like the HDAB to combine with the data you are applying for, you will receive instructions on how to deliver the data securely after the permit has been granted.

Why: Provide details on datasets the applicant already possesses or obtained from other sources outside the EHDS infrastructure. This information ensures the HDAB can evaluate the compatibility and compliance of these datasets with the data they are applying for.

What: Provide the following details about the existing dataset(s) that applicant wants to have combined with the dataset(s) they are applying for.

- What is the country/countries of origin* (max 500 characters).
- Who is the data holder(s) *(max 500 characters).
- Describe the database(s)/registry/registries*(max 500 characters).
- Describe the dataset record(s)/register/registers*(max 500 characters).
- To the extent applicable* (max 500 characters):
 - dataset
 - number of the files
 - format of the files

- size of the files
- special notes (are there direct/indirect identifiers, data should be pseudonymised, etc.).

Share any other data permits issued for the same project within the EHDS infrastructure or outside the EHDS infrastructure. The permits must be valid at the time data are processed. In this case provide the following information:

- Issuer, date of issue, expiry date, identification information.
- If the datasets were collected under prior authorisations (e.g., ethical assessment, research ethics approvals) or participant consent, attach the relevant approval documents and/or blank templates of the consent forms or information sheets. Do not include filled-in forms containing personal information, only blank templates.

Examples:

- Combining real-time data from wearable devices (e.g., heart rates, activity levels) collected by a current project with historical health data from a completed research project. This integration will allow enhanced analysis of cardiovascular health, enable longitudinal studies by providing continuous monitoring and help develop personalised health recommendations based on comprehensive insights.
- Genomic study combining DNA sequencing results from a biobank with electronic health records to investigate correlations between genetic markers and treatment outcomes for chronic diseases.

Tips:

- Plan ahead: Applicants shall consider this section thoroughly because if they decide later to combine additional data with the data authorised by the HDAB, they need to submit an amendment application to the same HDAB, incurring additional fees.
- Applicants shall use this section to inform the HDAB if they want to use other data in their possession to be combined with the datasets they are applying for.

6.4.8 Data processing, data protection and safeguards to prevent unauthorised use of data – Form Section 8

About this section

According to the EHDS regulation **Article 73(1)**, the HDABs shall provide access to electronic health data only through an SPE.

Why: Given the need for high-level protection when processing health data, the technical requirements for the infrastructure where the data will be worked with, once access is granted, are addressed in this section of the application form. HDABs will provide access to (individual-level) electronic health data exclusively through an SPE. The technical specifications like hardware specifications and analytical software needs may vary based on, e.g., data formats, analyst skills/preferences and planned analyses. Specifying computational needs in advance helps the HDAB identify the SPE best fit for purpose. As the GDPR applies for the sharing of personal electronic health data, this section also includes questions to ensure compliance with GDPR requirements.

What: Applicants must provide details on the below.

- The technical requirements for the SPE environment* (max 2000 characters):
 - Computational needs (e.g., RAM, processors, storage, software).
 - If there is a need to store the data after processing, indicate here the period of inactive data storage.
- The SPE provider (if known), including name and web address (max 2000 characters).
- Time frame for data to be available in the SPE for processing:
 - Specification of when they need access to the data considering the project schedule.
 - If the application involves datasets from multiple permits, ensure the timelines align to optimise workflows and avoid delays (for additional details on data processing see 8.8)
- Time frame for inactive data storage (archivation), following the period of processing and not exceeding the stated time in the permit (**Article 73(1) point e**).
- How the applicant complies with the principle of data minimisation when processing the data in the SPE * (max 500 characters)
- Data transfers outside the EU/EEA
 - Indicate if individual-level data will be transferred outside the EU/EEA, including through remote access.
 - Note: Even accessing data from a third country is considered a transfer and must comply with Chapter V of GDPR. In case of doubt, applicants all refer to their DPO.
- The regulation prohibits data controller identification * (max 500 characters)
 - Identify the data controller for the processing of electronic health data based on the application.
 - For individuals, this is the applicant.
 - For organisations, it is the organisation itself.
 - Note: Under the EHDS regulation, HDABs retain overall control of the shared data, ensuring its use complies with regulatory safeguards.
- Individuals who will need access to the SPE *:
 - Provide complete list of all individuals who will require access to the dataset in the SPE. Only these individuals will be included in the data permit if the application is approved.
 - This list can be updated later however this will require an amendment to the access application.

The applicants will be asked to confirm various statements documenting their commitment to key data protection obligations, including security. These statements are based on applicable EU laws, which are enforceable and auditable. By ticking these checkboxes, applicants confirm their understanding and acknowledgment of the obligations and prohibitions that will apply if their application is approved. These obligations are enforceable under applicable EU laws, including the GDPR and the EHDS regulation. While ticking the checkboxes does not create a new legally binding contract, applicants remain subject to all legal and regulatory requirements related to the use of health data.

Examples:

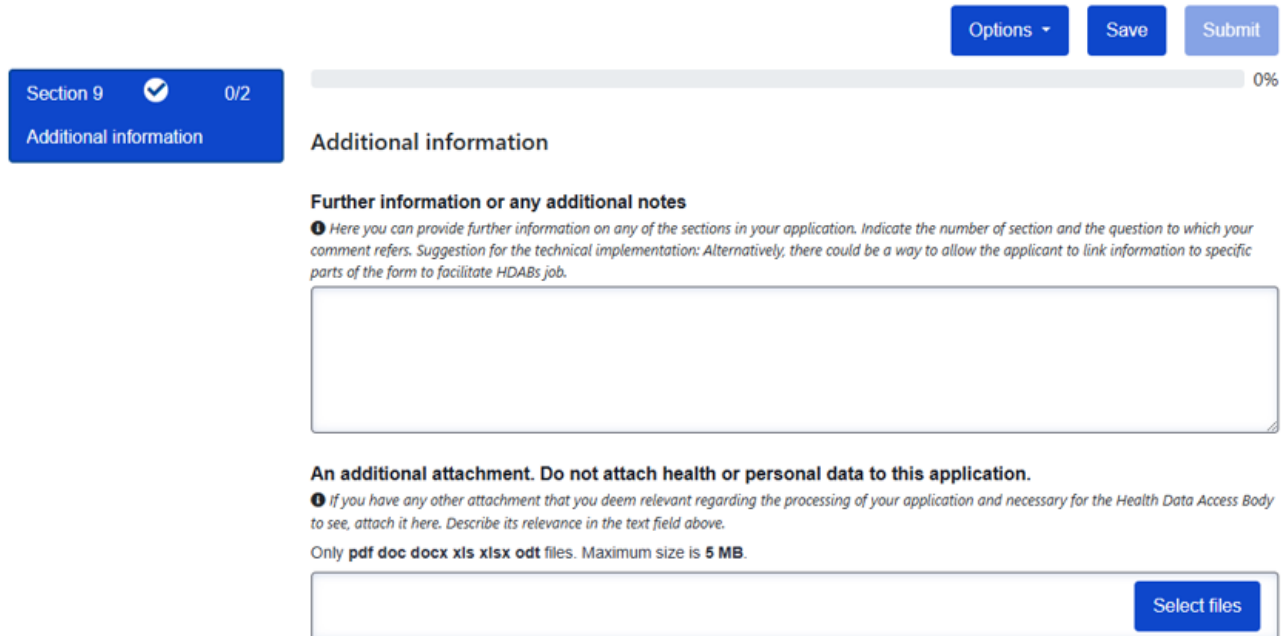
- Analysing omics datasets may require more computing resources, whereas basic statistical analyses on tabular data may need less, ultimately affecting the associated fees and potentially the choice of the SPE.
 - E.g., tabular data analysis may require fewer resources, such as 8GB RAM and standard statistical software like SPSS or Stata.
- Machine specifications depending on the complexity and volume of applicant's operations; applicants shall specify the following characteristics based on the scope and technical complexity of their planned analysis.
 - RAM: the memory needed for applicant's analysis. Intensive data processing often requires more RAM.
 - Processing Units (CPU/GPU): the number and type of processors required, especially for computationally demanding tasks.
 - Storage: adequate space for both data and software, particularly for large datasets.
 - Operating system: Linux, Windows or other operating system compatible with the software and tools to be used.

Tips:

- Applicants shall be precise and detailed in the description of desired SPE, including statistical software (e.g. SPSS, Stata, R), programming languages (e.g., Python, R), IDEs and notebooks (e.g., JupyterLab, RStudio), image visualisation and annotation tools (e.g., Cytomine, OHIF Viewer) and documentation tools (e.g., Microsoft Office, Adobe Acrobat Reader), among others.
- It is essential to identify applicant or their organisation's role under the GDPR and the related obligations. Refer to EDPB guidance on data controllership [here](#). If the applicant is affiliated with an organisation, they shall seek support from their DPO.
- If the applicant needs the data to be made available later, they shall indicate it in this section. Applicant may request to receive the data later, e.g., to coordinate the arrival of datasets from multiple permits into the SPE or to better align data availability with their project timelines.
- Data transfers – applicants shall determine if their project involves transferring individual-level data outside the EU/EEA and understand the obligations under [Chapter 5](#) of the GDPR. Accordingly, they shall consult their DPO or legal team early if they have any doubts or questions.
- Regarding inactive data storage, applicants shall plan for data retention requirements and justify the duration for inactive data storage.
- Applicants are encouraged to share their SPE requirements on the data processing details with HDAB in advance of the data access application submission to be sure that they are paying for services they really need.

6.4.9 Additional information – Form Section 9

Figure 6: Screenshot of the complete Section 9 of the application form.



Options Save Submit

Section 9 0/2

Additional information

Additional information

Further information or any additional notes

Here you can provide further information on any of the sections in your application. Indicate the number of section and the question to which your comment refers. Suggestion for the technical implementation: Alternatively, there could be a way to allow the applicant to link information to specific parts of the form to facilitate HDABs job.

An additional attachment. Do not attach health or personal data to this application.

If you have any other attachment that you deem relevant regarding the processing of your application and necessary for the Health Data Access Body to see, attach it here. Describe its relevance in the text field above.

Only pdf doc docx xls xlsx odt files. Maximum size is 5 MB.

Select files

Why: The application form was designed in a way to be as comprehensive as possible, using a combination of pre-defined response options (for the sake of consistency) and free text fields (for the sake of flexibility and explainability) to ease the work of the HDABs when evaluating the responses. Yet, applicants may still feel that they have relevant information about their project which, if they could share it with the HDAB, increases the likelihood of approving their application and gaining access to the data. While responding to these questions is not mandatory, applicants are free and encouraged to provide any such additional information to support understanding the intention of their application, its desired outcome and approval. Providing this information, though optional, may strengthen the applicant's case by clarifying unique project requirements, justifications or existing agreements with other stakeholders.

What: Anything that is relevant, necessary or complementing the application without repeating information already provided, in the form of free text (max 2000 characters) and an optional document upload. Applicants shall identify the relevance of the information, in addition to the section and question number that they are amending or explaining further.

Examples:

- Further information on applicant's planned data extraction order.
- More information about the data to be combined.
- Any agreements made with other parties (e.g., data holders) regarding data extraction.
- Additional reasons for choosing a particular SPE provider, including technical, security or financial considerations.

Tips:

- Applicants shall be concise and relevant: Provide only information that adds value to their application without overloading it. Ensure that they supply just enough information to explain the project's needs effectively, but don't overload the HDAB.
- Align with previous sections: Ensure that any additional information complements and expands upon earlier form responses rather than duplicating them.
- Anticipate questions: Consider areas where the HDAB might require further clarification or where additional context could enhance the understanding of the application.

6.4.10 Confirmation of information – Form Section 10**About this section**

Before this application is processed, you as the applicant must approve processing fees. To add information on the prices and the maximum price estimate for data extraction once available.

Why: To ensure that the applicant is aware of the potential financial consequences of submitting the application and that they confirm the accuracy of the provided information is recorded - which, in case of providing false and misleading information intentionally, may have an impact on the sanctions to follow.

What: The statements to be confirmed focuses on 2 aspects.

- Financial implications
 - Applicants shall be aware that application fees are calculated based on administrative and technical costs incurred by the HDABs. Even if the application is withdrawn after submission, fees for services already provided (e.g., dataset preparation, review) may still be charged.
- Accuracy of information
 - Inaccurate, incomplete, or false information may result in:
 - delays in processing the application;
 - refusal of the application;
 - penalties, including fines or legal actions, depending on the severity of non-compliance.
 - The HDABs are mandated to monitor and supervise compliance, including reviewing the accuracy of the application and the implementation of the respective data permit.

Tips

- While it is foreseen that after many hours of work invested into completing the form, applicants would like to submit it as soon as possible, the weight of confirming these statements should not be underestimated. Applicants shall take time, if necessary, to double-check the completeness and accuracy of the responses provided.
- Applicants shall familiarise themselves with the fee structure and non-compliance penalties to avoid unexpected financial or legal consequences. Get more information about this topic with the TEHDAS2 guideline D4.1 [8].

- Applicants shall be transparent – if there is uncertainty about any information (e.g., technical requirements or project timelines), they shall provide a clear explanation or note to avoid misunderstandings.

7 Terms and conditions of use

Structured procedures and conditions of the EHDS regulation are designed to facilitate the responsible and secure use of electronic health data (EHD) within the EHDS, balancing the benefits of data utilisation with the imperative of protecting individual privacy and data security. This requires the applicant to comply with certain terms and conditions when being granted access to data. These terms and conditions are binding before any access to electronic health data is granted.

7.1 General obligations and prohibitions

Article 61 of the EHDS regulation and other provisions set out further duties and obligations of data users in the context of using data. These requirements generally apply to data permits, as well as approved health data requests.

Table 1: Overview of health data applicant's expected behaviours and actions (do's) and the prohibited or discouraged behaviours (don'ts).

DO (=shall)	DON'T (=shall not)
• As a data user you are obliged to cooperate with HDABs during the processing of the application/request clarification (e.g. purpose of the use of data, the sampling method, responsibilities etc.) • While being granted access, you must keep contact and user information updated. • As data user you have to comply to high level of data protection and safeguards to prevent unauthorised use of data. • As data user, you need to ensure relevant expertise and adequate technical measures to comply with Art 68. • As data user you are obliged to inform HDABs of any significant findings related to the health of natural persons.	• As a data user you need to be aware that any processing of EHD beyond the permit's scope, is illegal. Any use of the data beyond the permit will require a new permit. • As a data user you need to be aware that any attempt to re-identify the natural persons from the dataset, is prohibited. • Any attempt to download person identifiable data from the SPE without specific agreement/permit from the HDAB, is prohibited. Replicating and attempts to replicate the data in any way is prohibited. • As data user you are not allowed to connect to the SPE from outside the EU, as this is transferring data to a 3rd country. A few exemptions can be permitted following Art 87 and elaborated in recital 94. • As a data user, your access credentials, (username/password, etc.) are personal and confidential. You are not allowed to share such

• As a data user you are obliged to inform the HDAB of your results obtained, e.g., publications etc. • As a data user you are obliged to make public results or output within 18 months of your data permit/access or the data processing period. If you need more time, you need to apply for/request extended time. When presenting and publishing, you are obliged to acknowledge the sources of the EHD and the fact that the EHD have been obtained in the framework of the EHDS. • As data user you are obliged to request an updated permit when needed, including approval of data access to a new user (Art 61(2), Art 68(12)). • As a data user you need to be familiar with the provisions of the EHDS regulation and GDPR when handling person identifiable data. In case of errors in the access process, you need to inform the HDAB. • As data user you can use only software tools allowed within the SPE (Art 73(5)). • As a data user, you should inform the HDABs when the dataset has been enriched and quality improved, to support continuous of the Data Source. In such cases you should make the improved dataset available for the original data holder, free of charge. • As a data user you are granted access to data based on your application/request. You are not entitled to refunds should the received data not meet your expectations.	information to anyone. Access is individual and actions monitored. Access to EHD is limited to those authorised and mentioned in the data permit
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Given the sensitivity of EHD, health data users do not receive unrestricted access to such EHD even in the data permit. All secondary use access is done through an SPE. Only non-personal EHD may be downloaded by the health data users from the SPE. To ensure this, downloading needs to be specifically requested from the HDAB, and only data specifically permitted may be downloaded.

Examples:

- While processing data in the SPE, the data user observes factors which substantiate a diagnosis of lung cancer that has not yet been recorded. The applicant must inform the HDAB, providing relevant facts, to allow the HDAB to and take the necessary steps (**Article 61(4)**).
- When obtained access to the data user must publish the anonymous results or outputs within 18 months of the completion, the processing in the SPE (**Article 61(4)**). The same goes for anonymous data accessed by request. In all publishing/reports, the Data Holder(s) and the EHDS is to be acknowledged. If under **Article 68(14)**, the Commission adopts an implementing act with a logo for acknowledging the contribution of the EHDS, the usage of this logo is compulsory. Should 18 months be too short, the data user needs to apply for / request an extension of period, which may be granted (see in 7.5).

7.2 Data permit

Data permit is “an administrative decision issued to a health data user by a HDAB to process certain electronic health data specified in the data permit for specific secondary use purposes, based on conditions laid down in Chapter IV of this Regulation” (**Article 2(2) point v.**). It is a formally binding decision issued by the HDAB as a designated body that allows access to certain data for secondary use and determines the terms of this use, in accordance with EHDS requirements. It has legal consequences for the applicant and defines a right to access and an obligation to comply with conditions.

A data permit (**Article 68**) grants a health data user the right to process specific electronic health data for defined secondary use purposes. In order to provide such access, the HDAB also instructs the data holder to make the specified data available.

Data permits are limited in time (up to 10 years), scope and purpose, with all terms and conditions outlined in the permit. If the permitted use needs to be adjusted, the health data user must request an amendment to the permit.

7.3 Agreement with data holders

In certain circumstances, data access may affect intellectual property rights or trade secrets (**Article 52**). The dataset description may indicate the existence of intellectual property rights (IPR) or trade secret restrictions related to a given dataset. However, the full scope and conditions of such protections may not always be known or disclosed at the metadata level. The data holder may raise additional IP or confidentiality concerns during the access negotiation phase, when the HDAB formally contacts them to process the data access request.

In case of such circumstances, the application might be rejected, or the HDAB might need to add specific conditions. Further, it may be necessary to instigate direct contractual arrangements between health data holders and health data users to enable the granting of access. To facilitate such agreements, the Commission will provide non-binding model contractual terms for use by stakeholders (**Article 52(4)**). **Article 52(5)** gives the health data

user as well as the health data holder the right to lodge a complaint against the refusal of the permit by HDAB, in accordance with **Article 81**.

Example: Data is needed for a study on the effectiveness of a new cancer treatment. Sensitive information from clinical trials conducted by a biotech company is included. A direct contractual agreement is required to ensure the protection of intellectual property while access to anonymised data is allowed.

7.4 Approved data requests

While data permits provide access to person identifiable EHD, the aim of a health data request is to receive anonymous data. Approval of data requests is also an administrative decision by the HDAB. However, this only allows receipt of data in an anonymised statistical format.

According to **Article 69**, the health data applicant may submit a health data request for the purposes referred to in **Article 53**. If approved, the HDAB provides anonymised statistical data as a response. Health data users must follow general obligations and maintain safeguards to prevent misuse, see [General obligations and prohibitions](#).

7.5 Making outcomes and results available

According to **Article 61(4)**, data users are required to publish the results or output of the secondary use of electronic health data (EHD), including information relevant for the provision of healthcare. The results or output of secondary use shall contain only anonymous data.

Health data users should inform the HDABs from which a data permit was obtained about the results or output of secondary use and assist them to make that information public on the HDABs' websites. Such publication shall be without prejudice to publication rights in scientific journals or other scientific publications.

Publication must acknowledge the sources of the electronic health data and the fact that the electronic health data have been obtained in the framework of the EHDS. For more information on handling research outcomes read [3].

7.6 Fees

Based on **Article 62** of the EHDS regulation, the HDAB shall inform the applicant of the expected fees that will be charged to assess the application, prepare the data and provide the data in a SPE for its processing. A processing fee will be charged for processing the application and will also apply in case of a cancelled application or a negative decision.

In multicountry applications or requests, each country will send their fees and the applicant will have to pay each of them separately in the country's respective platform.

It is necessary to mention that member states may establish **reduced fees** for certain types of health data users such as public sector bodies or Union institutions, bodies, offices and

agencies with a legal mandate in the field of public health, university researchers or microenterprises (**Article 62(1)**).

Please note that a data user is *not* entitled to a refund should the data not meet the expectations. More details about fees are available in the dedicated TEHDAS2 guideline [8].

7.7 Liability and sanctions

HDABs monitor the data user's compliance with the terms of the data permit (**Article 63(1)-(3)**) and are authorised to:

- revoke permits in cases of non-compliance;
- impose penalties to data users and data holders, such as fines or restrictions on future access.

Health data users can appeal against the HDAB decisions in accordance with national procedures. More details about penalties are available in the dedicated TEHDAS2 guideline [8].

7.8 Data quality

Health data user should be aware that the HDAB will make available electronic health data collected from various stakeholders of the health and care sector. The health data holder is required to keep the dataset description up to date at a minimum every year (**Article 60(3)**). If a dataset has been collected with public funding, the health data holder is required to also provide a data quality label (**Article 78**). The label includes details like where the data came from, how accurate it is and how it was processed.

Data user should be aware that data are being collected and curated for diverse purposes. Thus, it cannot be guaranteed that such data:

- are coherent in terms of formats, coding, nomenclatures and other criteria;
- fully comply with the specific needs of the health data user;
- meet the data quality expectations of the health data user.

This applies even when this is not specifically mentioned in the data permit.

Data users are responsible for assessing and managing data quality issues during their analysis.

The ongoing Quantum project⁴ aims to develop a data quality and utility label as regulated in **Article 56** which will facilitate access to high quality data.

7.9 Dataset completeness

The EHDS regulation introduces the right to opt out from the processing of personal electronic health data for secondary use (**Article 71**). When natural persons exercise this reversible right by explicit statement that they do not wish to have their personal electronic

⁴ <https://quantumproject.eu/>

health data processed for secondary use, their health data shall not be made available or otherwise processed after the natural person has exercised the right to opt out. This relates to the data permits issued under **Article 68** and health data requests under **Article 69** approved after the natural person has exercised the right to opt out. More information on how the Health Data Access Bodies are supposed to implement opt-out from the secondary use of health data is available in [9].

However, a member state may provide in its national law for a mechanism to make data for which a right to opt out has been exercised available, provided that all the following conditions are fulfilled:

(a) the health data access application or health data request is submitted by a public sector body or a Union institution, body, office or agency with a mandate to carry out tasks in the area of public health, or by another entity entrusted with carrying out public tasks in the area of public health, or acting on behalf of or commissioned by a public authority, and the processing of those data is necessary for any of the following purposes:

(i) the purposes referred to in 5.4 a), b) and c) (**Article 53(1) points a., b. and c., respectively**);

(ii) scientific research for important reasons of public interest;

(b) those data cannot be obtained by alternative means in a timely and effective manner under equivalent conditions;

(c) the health data applicant has provided the justification of exception from the right to opt out referred to in the EHDS regulation **Article 68(1) point g.**, or in **Article 69(2) point g.**

8 Expected timeframes from data application to data access

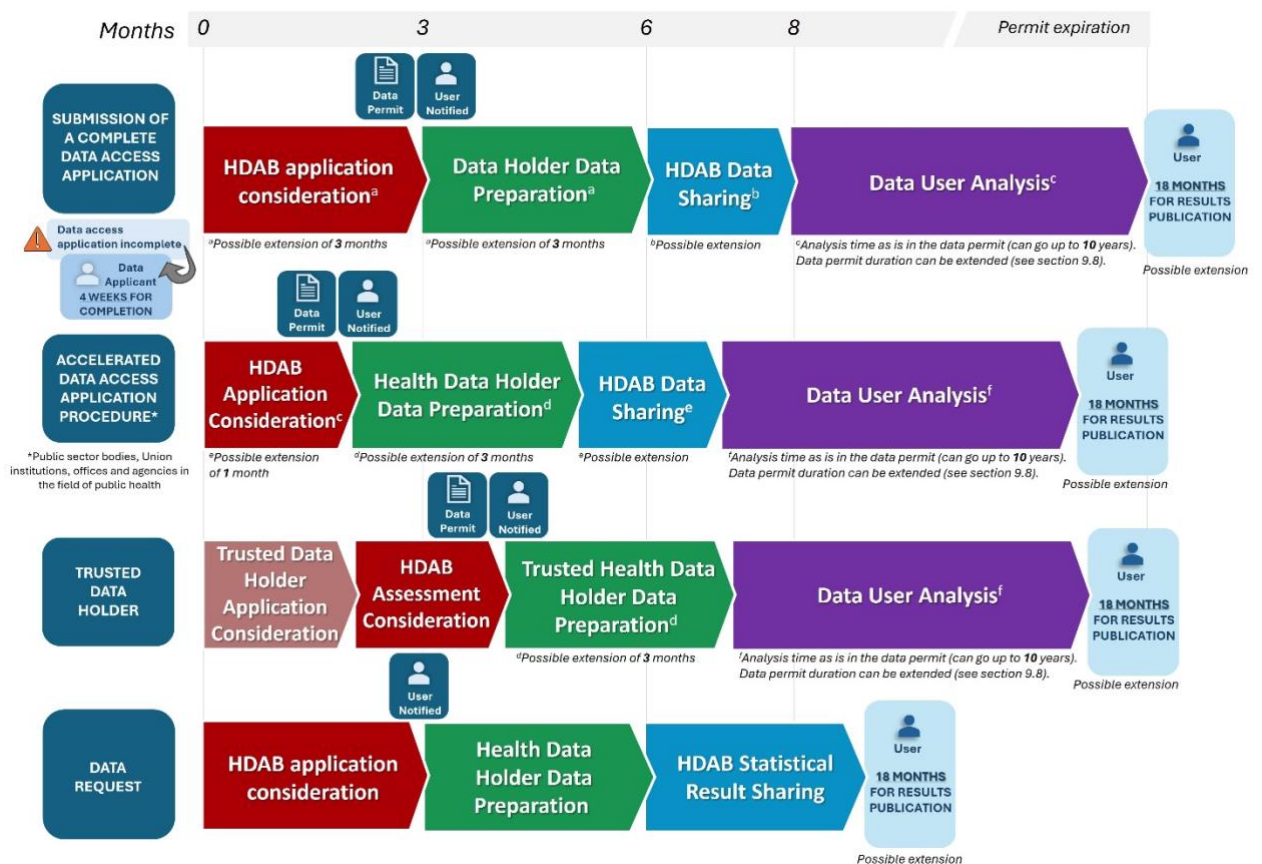
There are several procedures to get from the data application submission to results publication.

Figure 7 illustrates and describes the timeframes for each procedure. The specific procedure which the applicant can follow, depends on the chosen dataset(s) from the EU dataset catalogue and the responsiveness of all stakeholders involved in the process, including the HDAB, the (trusted) health data holder and the applicant.

Please note that these timeframes cover the indicated timeframes mentioned in the regulation. The actual time taken can be different from the maximum timeframes indicated in Figure 1

Figure 7. For example, data preparation by data holder can take up to 3+3 months but it could also take only 1 month.

Figure 7: Illustration of the various workflows within the HealthData@EU infrastructure, detailing the processes for both national and cross-border data requests. It outlines the expected timeframes and duration for each step in these workflows, providing a clear overview of the procedural stages involved.



In a standard data access application, the HDAB has 3 months to issue a permit, the data holder has 3 months to prepare the data and then the HDAB has 2 months to make the data available in SPE. Thus, according to this indicative timeline in case of a standard data access application, the data user will have access to electronic health data in the SPE 8 months

upon completing a successful data access application. The data user has then the time specified in the data permit to analyse the data in the SPE (8.8). After concluding data analysis or after data permit finishes, the data user has 18 months to publish the outcomes (7.5).

Exceptions may apply to the data holder's obligation to provide the data in 3 months. For example, the obligation to provide data does not extend to persons who have opted out. In justified cases, the HDAB may extend the deadline for the data holder by a maximum of 3 months.

However, the health data holder may not refuse to provide the data on the grounds to intellectual property (IP) protection. In such circumstances, additional guarantees may be required.

For public sector requests, the HDAB has 2 months for application consideration. If a trusted data holder is involved, the steps differ slightly, but the overall timeframe is similar.

For data requests, the HDAB has 3 months for decision-making, the data holder has 3 months for data preparation and statistical results are shared within another 3 months.

In all scenarios, the data user has 18 months to publish the analysis' outcomes. Additionally, there are 4 weeks to complete the application form if requested by the HDAB. A complete application may lead to data access in less than 8 months.

Chapter 6.4.6 covers the description of data needed. Note that the applicant has the possibility to indicate multiple extractions of the same dataset in time (e.g., when subsequent years of a single dataset are requested). In this case, this will already be considered upon assessing the request and the steps indicated in

Figure 7, covering the application consideration and assessment, which will not be repeated.

8.1 HDAB application consideration

Once a data access application is submitted to the relevant HDAB, the application consideration phase begins. For complete applications, where no further information is required from the applicant, the HDAB has a time limit of 3 months to consider and respond to an application by either issuing a permit or rejecting the application (**Article 68(4)**). The consideration phase can be extended to a maximum of 6 months, when more time is needed by the HDAB to examine the application and after timely communication to the data applicant.

8.2 Accelerated procedure

In some cases, an accelerated procedure is applicable (**Article 68(6)**). Note that this accelerated procedure is only applicable for public sector bodies and Union institutions,

bodies, offices and agencies with a legal mandate and only for specific purposes outlined in **Article 53(1), points (a.), (b.) and (c.)** and referred to in **recital (75)**.

This accelerated procedure shortens the maximum allowed time of 3 months to 2 months for an HDAB to issue a permit or reject the application. Nevertheless, under specific circumstances, the HDAB still has the possibility to extend the assessment period to a maximum of 3 months, thus extending the period with 1 month to evaluate the application.

8.3 Applications involving trusted health data holders

If a trusted health data holder (HDH) is involved, the steps differ slightly, but the overall timeframe is similar (see

Figure 7). In applications that cover electronic health data held by trusted health data holders these are responsible for evaluating the data access application and issuing a positive or negative assessment up to 2 months after receiving the data access application forwarded by the HDAB. The assessment made by the trusted health data holder will be sent to the HDAB, that has 2 months to evaluate the assessment made by the trusted health data holder. The HDAB is the ultimate instance responsible for issuing the permit (in case of a positive assessment) or rejecting the application, as well as for informing the applicant of the outcome of its application. Under this procedure, the consideration phase takes up to 4 months in total. In case a permit is granted, the trusted health data holder will prepare the data (as described in 8.6) and make them available for use on its SPE, with no intermediation by the HDAB (the step 8.7 does not apply). The conditions for data user analysis are as described in 8.8 .

It is legally possible for a trusted HDH to be involved in the assessment of an application submitted under the accelerated procedure (see 8.2). In such cases, the accelerated timeline of two months (extendable by one month in complex cases) would still apply to the Health Data Access Body (HDAB). Since the trusted HDH has up to two months to make a proposal for decision under **Article 72(5)**, the HDAB and trusted HDH would need to coordinate closely to ensure compliance with the shortened deadline.

8.4 Additional information may be asked by HDABs

In case the information in the application is deemed incomplete or is not of sufficient detail, the HDAB can request additional information from the applicant. When additional information is requested, the applicants have 4 weeks to respond to the information request by the HDAB. If the applicant fails to provide additional information within this 4-week maximum timeframe, the application may be rejected. If the application is rejected, the applicant will still be subject to pay any fees incurred during the process. Please note that the standard 3-months for HDAB assessment only start counting after the application is considered complete.

8.5 Decision on data use permission

HDABs are tasked with deciding on health data access applications and issuing administrative acts, i.e. data permits. The table below provides a summary of the possible HDAB decisions and the next steps relevant for the HDAB and applicant.

Table 2: HDAB decisions with possible scenarios as the next steps.

HDAB decision	Next step(s)	
	HDAB...	Applicant...
Applicable fees	• Informs Applicant about the fees covering the overall processing of the electronic health data within the EHDS infrastructure (Art 62(5)) – for more details about fees see TEHDAS2 Guideline [8]	• Decides whether to accept or withdraw the application, e.g. due to lack of funding available to cover all fees. If the applicant withdraws the application, s/he shall only be charged the costs that have already been incurred.
Issues a data permit	• Publishes information about the permit granted within 30 days of the issuance and maintains internal record about the permit. • Engages the health data holders and the SPE provider.	• Becomes “health data user” with all relevant rights and obligations.
Rejects the application	• Justifies its decision (Art 68(9)). • Publishes information about the rejection within 30 days and maintains internal record (Art 57(1) point j.).	• May resubmit the application, taking into consideration the HDAB’s justification. • May contest the decision via the established channels.
Provides a response in a different format	• May decide to provide a response in an anonymous statistical format if the Applicant agrees to that approach (Art 68(3)).	• May resubmit the application, taking into consideration the HDAB’s justification. • May contest the decision via the established channels.
Application is incomplete (Art 68(4))	• Notifies the applicant about incomplete application. • The application clock (HDAB assessment period) is not initiated until receiving the pending information from the applicant..	• Completes the application within 4 weeks, otherwise the permit is not granted.
	• If modifications to the permit are needed by either the applicant or the HDAB, a formal amendment process must be initiated.	

Revisions or amendments to the data permit	The amended permit will include updated conditions, data specifications or timelines, subject to regulatory requirements, and data processors with access to SPE.	
	Approval is at the discretion of the HDAB.	Applicants must provide a justified reason for the amendment.
Delayed decision	Notifies the applicant.	Applicant may escalate the process and present a complaint.

The applicant will be able to see the status of their application during the decision process.

Please note 🕒:

When the HDAB, during application assessment, finds that the application is incomplete, the data applicant will be requested to provide the extra information. This must be completed within 4 weeks. Please note that during the time the applicant is completing the information, the standard 3-months for HDAB assessment have not been initiated yet. The standard 3-months for HDAB assessment only start counting after the application is considered complete. Therefore, in cases where additional information is requested, a delay in the overall assessment time will occur, that will be as long as the time required by the applicant to send back the complete information.

As soon as the permit is issued, the applicant will be notified. The permit's template is pre-defined in the regulation and will contain the description of the data to which access is granted, the duration of access, technical characteristics and tools, specific conditions and the fees to be paid. The permit enables the health data user to process the data, as specified in the permit, for specific secondary use purposes based on the regulation's and the permit's conditions.

8.6 Data preparation

After the data permit is issued, the HDAB will immediately request the data from the health data holder(s). The data holders have 3 months to send the data to the HDAB and in some cases, this period can be extended by another 3 months (**Article 60(2)**). The data preparation period is reflecting the requirements of the data applicant written in the health data access application form. During this time frame the health data holders will be extracting the data following the data minimisation and purpose limitations rule (**Article 66**). Then, the HDAB will have 2 months to prepare it, link it, clean it, pseudonymise it and share it in the SPE. When datasets involve data from multiple countries, the same timeframes (up to 6 months total) apply for preparation. Coordination among HDABs does not extend the preparation

period, though applicants should anticipate potential complexities. Data preparation and processing by HDAB is described in appropriate TEHDAS2 guidelines [5, 6].

8.7 HDAB makes electronic health data available

The HDAB shall make available the electronic health data to the health data user in a SPE within **2 months** after receiving them from the health data holders, unless the HDAB specifies that it will provide the data within a longer specified timeframe (**Article 68(7)**). One of the TEHDAS2 Guidelines [10] on making electronic health data available provides specifics on the cases that can lead up to a longer timeframe. Note that there is no exact timeframe stated when going beyond the indicated 2 months. In any case this additional time needed should be reasonable and in line with the complexity of the data requested.

Once data is available in the SPE, the applicant will be notified. The individuals indicated in the application will be given access to the SPE accordingly.

8.8 Data user analysing data

Once the data is shared with the applicant, the data use phase starts. With the issuing of the data permit, the applicant becomes a health data user. HDAB makes the datasets comprised in the granted data permit available to the data user in the dedicated SPE.

Data users have the amount of time specified in the data permit to process the data in the SPE (this amount of time can be up to 10 years). If the health data user discovers during data analysis that additional time will be required, an extension of data permit's duration may be requested. This extension can be requested only once and the period cannot be longer than another 10 years. Extension requests need to be properly justified when presented to the HDAB and shall be provided one month before the expiry of the ongoing data permit. Please ensure to make your extension request to the HDAB with enough notice. The extension requires an amendment and may incur an additional fee.

After completing data analysis in the SPE, the health data user has 18 months to make the results publicly available and with the HDAB (see 7.5).

The data from the SPE can be on demand of the health data user, securely archived by the HDAB for reproducibility reasons enabling a reanalysis of the data if needed for instance in peer-review processes but no longer than the permit is lasting (**Article 68**).

Tip: When applicants apply for more datasets from multiple countries, they can specify in which SPE the data should be analysed already in the data access application form, see 6.4.8 and [2] for details on data use in a secure processing environment. Note that the HDAB still has the final decision on what SPE will be used to provide the data.

References

- [1] „D6.4 Data Access Application Management System (DAAMS) - Technical specification for health data access bodies“.
- [2] „D7.1 Guideline for data users on how to use data in a secure processing environment“.
- [3] „D8.4 Guideline for data users on handling research outcomes“.
- [4] „D4.3 Guideline for Health Data Access Bodies on international and third country access and transfer of electronic health data“.
- [5] „D6.3 Guideline for Health Data Access Bodies on the procedures and formats for data access“.
- [6] „D7.5 Guideline for Health Data Access Bodies on linkage of health datasets“.
- [7] „D7.2 Guideline for Health Data Access Bodies on data minimisation, pseudonymisation, anonymisation and synthetic data“.
- [8] „D4.1 Guideline for Health Data Access Bodies on fees and penalties for non-compliance related to the EHDS Regulation“.
- [9] „D8.1 Guideline for Health Data Access Bodies on implementing opt-out from the secondary use of health data“.
- [10] „D6.1 Guideline for data holders on making personal and nonpersonal electronic health data available for reuse“.
- [11] „D4.2 Guideline for Health Data Access Bodies on collaboration with other parties“.
- [12] „D5.1 Guideline for data holders on data description“.
- [13] „D5.2 Guideline for Health Data Access Bodies on minimum categories and limitations on the reuse of health data“.
- [14] „D8.3 Guideline for Health Data Access Bodies on informing natural persons about the use of health data - “Citizen Information Point”“.
- [15] „D8.2 Guideline for Health Data Access Bodies on implementing the obligation of notifying the natural person on a significant finding from the secondary use of health data“.

Glossary

Anonymised statistical format: An anonymised statistical format refers to aggregated data that does not include information on individual data subjects [7].

Anonymisation: Practices and techniques that help produce anonymous data. It ensures that the data cannot be used to (1) identify any individual directly or indirectly, (2) draw conclusions about a specific individual and (3) link data concerning an individual to other materials. Anonymised material must be impossible or extremely difficult to revert to an identifiable format that could be used to identify a specific individual. It is recognized that anonymisation processes could be reverted in the future and that anonymisation always involves a residual à re-identification risk. (Findata, producing anonymous results (accessed 10.04.2025); p.11-12 WP216; 10 Misunderstandings related to anonymisation)

Benefit: refers broadly to positive outcomes of data use. It encompasses social (e.g. giving more people access to education and information), health (e.g. improved medical treatment) and environmental (e.g. improved response to climate change), among others. Taken from: <https://pluto.univie.ac.at/glossary>

Health data access application: An application seeking to access individual-level electronic health data for secondary use in an anonymised or a pseudonymised format. *Health data access body (HDAB):* Member state-designated data permit authority that facilitates the secondary use of electronic health data. HDABs decide on health data requests and access applications, authorise and issue data permits, obtain data from data holders and make data available in Secure Processing Environments. HDABs systematically track the data request and data access applications received and the data permits issued. As per **Article 58** of the EHDS regulation, HDABs are required to publicly list information on the data permits issued.

Pseudonymisation: The processing of personal data in such a way that the data can no longer be attributed to a specific data subject without the use of additional information, provided that such additional information is kept separately and is subject to technical and organizational measures to ensure non-attribution to an identified or identifiable person. (GDPR Article 4(5))

Health data access body (HDAB): Member state-designated data permit authority that facilitates the secondary use of electronic health data. HDABs decide on health data requests and access applications, authorise and issue data permits, obtain data from data holders and make data available in Secure Processing Environments. HDABs systematically track the data request and data access applications received and the data permits issued. As per **Article 58** of the EHDS regulation, HDABs are required to publicly list information on the data permits issued.

Personal electronic health data: data concerning health and genetic data, relating to an identified or identifiable natural person, processed in an electronic form. (ref. EHDS article 2 – 2(a))

Non-personal electronic health data: electronic health data other than personal electronic health data, including both data that have been anonymised so that they no longer relate to

an identified or identifiable natural person (the 'data subject') and data that have never related to a data subject. (ref. EHDS article 2 – 2(b))

Electronic health data: personal or non-personal electronic health data.

Health data request: The applicant may submit a health data request for the purposes referred to in **Article 53** of the EHDS regulation with the aim of obtaining an answer only in anonymised statistical format upon the HDAB's administrative decision (approval).

Health data applicant: A natural or legal person submitting a health data access application for the purposes referred to in **Article 53** of the EHDS regulation to the HDAB.

Health data holder: An entity that processes electronic health data as a data controller for the purposes of provisioning care or healthcare, developing healthcare products, services, wellness applications and undertaking healthcare research, or processing healthcare data for innovation, policy development, official statistics or patient safety or for regulatory purposes.

Health data user: A natural or legal person, including European Union institutions, bodies or agencies, which has been granted lawful access to electronic health data for secondary use pursuant to a data permit, health data request approval or an access approval by an authorised participant in HealthData@EU etc.

Data permit: An administrative decision issued to a health data user by a HDAB to process certain electronic health data specified in the data permit for specific secondary use purposes based on conditions laid down in Chapter 4 of the EHDS regulation (Article 2(aa)).

Data minimisation: A principle identified in [Article 5](#) of the GDPR, mandating organisations to only collect, store and process personal data that is absolutely necessary to achieve their specific purpose.

Data controller: Identified by [Article 4](#) of the GDPR as any natural or legal person, public authority, agency or other body that determines the purposes and means of processing personal data. The controller is the party that determines why the processing of personal data takes place and how the processing will take place.

Legal basis of data processing: Identified by [Article 6](#) of the GDPR as the conditions under which personal data processing is considered lawful. And for the purposes included in EHDS regulation, the processing is based on "public interest".

Public value: Public value is a weighted composite of risks and benefits of the data use. This takes into account the sustainability of benefits, addressing future societal needs, distributing benefits fairly, evaluating potential harm, ensuring stable safeguards through risk assessment, and correcting any harms that may occur. Taken from: <https://pluto.univie.ac.at/glossary>

Secure Processing Environment (SPE): Defined in **Article 73** of the EHDS regulation as an environment in which access to electronic health data can be provided in following a data permit. An SPE is subject to technical and organisational measures and security and interoperability requirements. Specifically allowing access to only those people listed in the permit, as well as user authentication, authorisation, restricted data handling, logging and the compliance monitoring of respective security measures.

Transfer of data outside the EU/EEA: Defined in [Chapter 5](#) (Article 44 to 50) of the GDPR as general principles, adequacy decisions, appropriate safeguards and specific derogations for transferring personal data to third countries or international organisations. The European Data Protection Board (EDPB) identifies [three cumulative criteria](#) to identify a transfer outside the EEA:

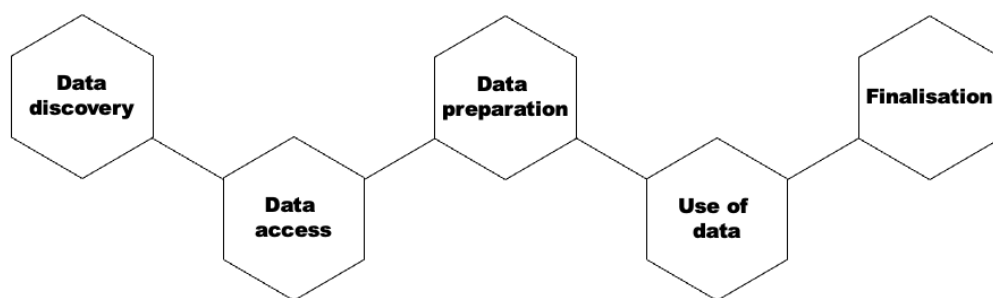
- a controller or a processor is subject to the GDPR for the given processing;
- this controller or processor discloses by transmission or otherwise makes personal data available to another organisation (controller or processor);
- this other organisation is in a country outside EEA or is an international organisation.”

Trusted health data holder: Member State designated health data holder for whom a “simplified” procedure can be followed for the issuance of data permits as defined in Article 72 of the EHDS regulation. Trusted health data holders leverage their expertise on the data they hold to assist the HDAB by providing assessments of data requests or access applications. Once data permits are authorised, these trusted data holders provide the data within an SPE that they manage.

Annex 1: User journey

When a data user applies for electronic health data for secondary use purposes, such as research and innovation activities, education and policymaking, within the EHDS, the user journey consists of several stages (see Figure 8). Access for certain purposes (public or occupational health, policy-making and regulatory activities and statistics) is reserved for public sector bodies and Union institutions (see Chapter IV, Art. 53(1) and 53(2)).

Figure 8: EHDS user journey consists of five main phases: data discovery, data access, data preparation, use of data and finalisation.



Data discovery

Before being able to use the data, the user needs to investigate whether the data needed is available and whether it is available in the necessary format for the secondary use purpose. This phase is called data discovery. Datasets available in the EU can be found in a metadata catalogue at [HealthData@EU Central Platform](#). Once the data discovery is completed, the user can begin the process of applying for the data.

Data access

In the data access phase, the user fills in and submits a dedicated and standardised data access application form or a data request to a HDAB. The user must complete the information required in the form, upload necessary documents and provide justifications as needed.

Data access application form is used when the user seeks to use personal level data. Data request is for cases when the user wants to apply for anonymised statistical data.

Data preparation

During this phase, the data holder(s) deliver(s) the necessary data to the HDAB, which starts to prepare the data for secondary use. Techniques for pseudonymisation, anonymisation, generalisation, suppression and randomisation of personal data are employed. The data minimisation principle (as per the GDPR) must be respected to ensure privacy.

Use of data

In this phase, the user performs analyses based on the received data for the purpose defined in the application phase. Analysing personal level data must be performed in an SPE. The duration of this phase is specified in the Regulation (**Article 68(12)**).

Finalisation

This last phase of the user journey concerns data user's duties regarding analysis outcomes derived from secondary use of data. Data user must publish the results of the secondary use of electronic health data within 18 months of the completion of the data processing in an SPE or of receiving the requested electronic health data. The results should be provided in an anonymous format. The data user must inform the HDAB of the results. In addition, the data user must mention in the output that the results have been obtained by using data in the framework of the EHDS.

Annex 2: Methodology

The contributors participated according to their promised commitments, ensuring a collaborative and thorough development process. See below the information about our structured work together.

- Working meetings
 - We conducted four working meetings to discuss and outline the key components and structure of the guideline, as well as address any unclarities in the regulation.
- Write-a-thons
 - Five write-a-thons, each lasting three hours, were held to collaboratively draft and refine the content. What was not written during the write-a-thon was finished offline by the contributor who was given the responsibility for it.
- Consultations with DG SANTE
 - Three meetings with a representative from DG SANTE were organised to ensure alignment with regulatory requirements and to gather expert feedback.

We still expect the continued involvement of both minor and major contributors after the public consultation to implement the feedback received.

Annex 3: Summary of the public consultation results and main feedback

A draft version of this document was in public consultation in January and February 2025. This document was commented in total for 87 times. The number of responses may contain some duplicates as there was no individual identification and verification required to respond to the surveys. Some respondents have also responded both from data holder's and data user's perspective. The responses came from 18 different countries from the EU countries and the European Economic Area countries. Responses from international organisations were missing. The respondents were primarily from three main types of organisations, listed in order of prevalence: public organisations, private organisations, and academic/research organisations.

To implement the public consultation feedback, we established small working groups for each chapter (Ch6-9), consisting of 3-4 members nominated by major and minor contributors, along with two observers. Feedback was received in an Excel file, with responses from individuals or teams in rows and chapter-specific comments in columns. Working group members reviewed the feedback offline, assessing relevance and feasibility for the guideline using semaphore colours:

- GREEN: Clear to implement – suggest how to reflect the comment via a TODO note.
- YELLOW: Uncertain – note what needs to be discussed with the working group.
- RED: Not to be considered – reasons described in the notes.

Five dedicated 3-hour meetings were held to discuss the yellow fields and collect all TODOs from the feedback. TODOs were addressed during meetings or offline by working group members. A finalisation meeting was held to discuss any remaining tasks.

In the provided answers were found comments and suggestions which we can divide into several major clusters/ topics:

Nomenclature

- Consistent nomenclature aligned with EHDS Regulation was requested.
- Clear expressions used for personal and non-personal data were ensured.

Clarity of the instructions

- Requests for more examples and clear instructions of the application process were received.
- More examples and three new figures to the guideline focused on the process of application, decision diagrams for the datasets searches and an overview of the application form, text adjusted.

Technical specifications and details on SPEs usage

- Interest in federated data analysis, SPE usage, and how to count the computational power needed for the data processing was noted.
- Links to the TEHDAS2 guideline on secure processing environments were provided.

Ethical and Legal Considerations

- Importance of ethical reviews, GDPR compliance, and data privacy and security concerns, including the Regulation's influence on informed consents were requested.
- Discussions on informed consent validity and data minimisation principles were incorporated in the guideline.

User experience enhancement

- Confusion about the application process, especially for multicountry datasets was noted.
- Text related to dataset availability was clarified, a decision diagram was added, and links to relevant information sources were provided.

Data quality and metadata description of the datasets in the catalogue

- Comments on data quality, how to specify variables of interest and how to get that information before application filling in were received. Recommendations and links to QUANTUM project were provided.

Inputs related to other components of the EHDS infrastructure (catalogue, HDAB, data holder, SPE) were conveyed to the TEHDAS coordinator for dissemination to the appropriate working groups. Where the scope of the guideline concluded, links to relevant TEHDAS2 guidelines and technical specifications were incorporated. The public consultation highlighted areas of the guideline that were confusing and incomplete, assisting in identifying critical details for both novice and experienced applicants. All topics were thoroughly reviewed, and the most pertinent requests were implemented. The feedback was appreciated, although evaluating all the suggestions was challenging.

Annex 4: Type of Input Fields in the Common Access Application Form

» Open or free text answers

Input from the applicant is required and no answer categories are specified. Applicants can provide their free text responses while obeying the remaining character limit, which will be displayed next to the question once the applicant starts typing.

Figure 9: Project name field of the data access application form.



» Single- and multiple-choice answers

For these questions, answer categories are specified from which either exactly one or several can (or must) be selected by the user. Applicants should select one or more predefined options according to the specification in the question.

Figure 10: Example of selection of an option from given list.

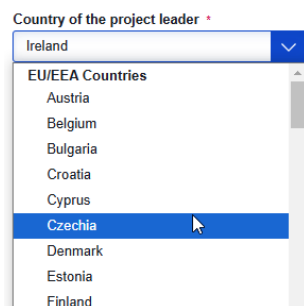


Figure 11: Example of multi-choice answer to a question in the form.

Select the option corresponding to your purpose of data use. Health data access bodies shall only provide access to electronic health data referred to in Article 33 where the intended purpose of processing pursued by the applicant complies with the following purposes (as per Article 34(1)) listed below). *

- ☒ a. Activities for reasons of public interest in the area of public and occupational health, such as protection against serious cross-border threats to health, public health surveillance or ensuring high levels of quality and safety of healthcare and of medicinal products or medical devices
- ☐ b. To support public sector bodies or Union institutions, agencies and bodies including regulatory authorities, in the health or care sector to carry out their tasks defined in their mandates
- ☒ c. To produce national, multi-national and Union level official statistics related to health or care sectors
- ☐ d. Education or teaching activities in health or care sectors
- ☐ e. Scientific research related to health or care sectors

» File uploads

It is possible/necessary to upload files/documents at certain stages of the form. The applicant should upload documents as required, such as ethical approvals or financial statements

Figure 12: Example of question with possibility to upload a file.

If the dataset records involve permits issued by other parties or they have been collected with consent, attach the permit documents here

Only pdf doc docx xls xlsx odt files. Maximum size is 5 MB.

» Automatic retrieval of information

At certain input prompts in the form, it is possible to retrieve data automatically. If this possibility is available, it is marked accordingly.

Figure 13: Example of yes/no answer possibility with button enabling filling in the answers from user profile.

Are you applying for data for carrying out tasks enshrined in the mandate of your organisation/institution? *

Tasks in your organisation's/institution's mandate mean tasks based on national or European Union law.

☐ Yes
 ☐ No

» Date specifications

Dates are entered via interactive input fields where the respective date can be selected in a calendar view

Figure 14: Example of date selection via interactive calendar.

If you need to store the data after processing, indicate here the period of inactive data storage

From To

November 2024

SUN	MON	TUE	WED	THU	FRI	SAT
27	28	29	30	31	1	2
3	4	5	6	7	8	9
10	11	12	13	14	15	16
17	18	19	20	21	22	23
24	25	26	27	28	29	30

for the EEA? *

three cumulative criteria to qualify a processing operation as a transj
r the given processing. 2) The exporter discloses by transmission or on
controller, joint controller or processor ("importer"). 3) The importer is
to the GDPR for the given processing in accordance with Article 3, or
ropa.eu/system/files/2023-02/edpb_guidelines_05-
f_the_gdpr_v2_en_0.pdf

e controller of the data to be formed based on this

rocessing personal data. In other words, the data controller decides thi
st person, for example a business, an SME, a public authority, an agen
ection guide/data controller data processor an